

VULNERABILITY PRACTICES AMONG PROFESSIONALS IN MULTIDISCIPLINARY SETTINGS

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Professional workers are expected to display confidence and mastery in their work roles, and may be hesitant to reveal their doubts and insecurities, especially in multidisciplinary settings characterized by status differences. However, scholars have recognized the benefits of sharing vulnerabilities to create opportunities for social support. We contribute to the literature by revealing *how* this might be achieved through a ritualized sequence of “vulnerability practices” in a dedicated setting. Based on a qualitative study of a weekly meeting where a multidisciplinary group of professionals shares experiences, we develop a process model that explains how vulnerability practices evolve through phases of individual disclosure and communal reflection bridged by theorizing from experiences. We reveal how a delicate tension between social pressure (to display vulnerabilities) and social support (to contain them) underpins these practices, and how this tension is influenced by power relations associated with multidisciplinary. We show how participants’ appraisals of past episodes shape their willingness to engage in subsequent episodes, reinforcing or undermining the capacity of the setting to serve as a “holding environment” offering interpretation and containment. We thus reveal holding environments at work to be precarious accomplishments, created and maintained through ritualized vulnerability practices, and imbued by power relations.

The authority of professionals such as physicians, nurses, lawyers, architects, consultants, and academics

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is intertwined with their claims to expertise, making it difficult for them to display the doubts and insecurities they experience in their daily work (Clarke & Knights, 2018; Kitay & Wright, 2007; Pratt, Rockmann & Kaufmann, 2006). For example, a key part of physician training is contained in the “hidden curriculum”—a socialization process through which medical residents learn to project strength, rationality, and self-control (Hafferty, 1998). In nursing, Steege and Rainbow (2017) speak of a “Supernurse culture” accompanied by a “cloak of invulnerability” that masks doubts and fatigue. Bourgoin and Harvey (2018) show how consultants feel the need to present an image of expertise and mastery to clients while scrambling to acquire the knowledge they need to meet expectations. In other words, professionals hesitate to display “vulnerability,” defined here as a pervasive experience of doubt and insecurity about work role performance. Displaying vulnerability in the course of one’s work may be viewed as counter-institutional

in that it violates established professional norms (Chreim, Langley, Reay, Comeau-Vallée & Huq, 2020).

The concern to show mastery and confidence and avoid displaying vulnerability may be particularly intense in multidisciplinary settings, where various professionals work together. This is because these settings are imbued with status and power differences based on institutionalized professional hierarchies (Abbott, 1988; Huq, Reay & Chreim, 2017), and they are also arenas for jurisdictional negotiations (Barley, 1986; Bechky, 2003). Displaying vulnerability to those of a different status may challenge a person's sense of self as a competent professional, potentially undermining their ability to maintain jurisdictional authority (Ahuja, 2023).

Although vulnerability intuitively implies weakness, some scholars have begun to conceptualize willingness to *share* one's vulnerabilities as a strength (Brown, 2015; Corlett, Mavin & Beech, 2019; Corlett, Ruane & Mavin, 2021; Hay, 2014). For example, Brown (2015) argues that acknowledging vulnerability reveals courage, humanity, and openness to learning. Others have suggested that vulnerability is a pervasive human experience that, when displayed among group members, attaches them to one another and strengthens commitment to the group (Creed, Hudson, Okhuysen & Smith-Crowe, 2022; Jordan, 2008). In this view, vulnerability displays can allow members to foreground their relational interdependence, enabling them to resist role expectations that they experience as detrimental to themselves and others (Butler, 2016; Fotaki, 2023).

While displaying vulnerability can be seen as a strength, it is also risky. Thus, some scholars have begun to explore settings within which vulnerability can be displayed to help people manage doubts and insecurities while limiting the risks. For example, vulnerability displays may be enabled by ad hoc dialogue with close colleagues (Hibbert, Beech, Callaghan & Siedlok, 2022), or in more formal settings such as leadership development workshops, or communities of practice (Corlett et al., 2021; Delgado, de Groot, McCaffrey, Dimitropoulos, Sitter & Austin, 2020). Interactions within such settings offer opportunities for people to engage in role-based learning and improve their ability to manage their doubts and insecurities at work (Corlett et al., 2021; Hibbert et al., 2022; Johnson & Milberg, 2020). Kahn (2001), building on Winnicott (1965), used the term "holding environments" to describe social contexts that allow participants to confront emotionally challenging issues, display their vulnerabilities, and receive support from others. Petriglieri and Petriglieri (2020), building

on Shapiro and Carr (1991), describe holding environments as providing "containment" (ability to manage difficult emotions like doubt and insecurity) and "interpretation" (ideas about how to understand these experiences).

Although literature suggests that certain social contexts may serve as holding environments, it is unclear exactly *how* they may do so. Creating a dedicated social setting where professionals can interact (e.g., a community of practice) is a useful starting point, but does not appear sufficient. There is a need to understand the specific *practices* through which holding environments are created and maintained within professionalized work settings to enable people to manage role-based vulnerabilities and reconsider role performances. This puzzle is of particular concern for multidisciplinary work, as power relations may "shape the extent to which holding environments can exist between members who represent different groups" (Kahn, 2001: 274). Multidisciplinary contexts thus complicate efforts to design holding environments, and more specifically to enact practices that enable members to express their vulnerabilities and receive support. Theorizing those practices is the focus of this paper.

This relatively unexplored focus is important because it sheds light on how professionals might normalize and even value vulnerability displays, enabling them to receive support, and potentially altering how they conceive of and enact work roles. A practice-based approach also enables us to conceptualize how and why engagement in vulnerability displays may benefit or harm participants. This leads to the following research question: *How and with what perceived effects do professionals in multidisciplinary settings engage in practices that enable the sharing of doubts and insecurities experienced in the performance of their work roles?*

Our focus on practices enabling vulnerability display emerged as we observed weekly meetings of a multidisciplinary team of primary health caregivers and noticed episodes in which participants shared doubts and insecurities related to dealing with complex patient cases. As our study progressed, we noticed that professionals engaged in ritualized practices through which they encouraged each other to make themselves vulnerable. We conceptualize these as "vulnerability practices" and unpack them and their consequences, examining whether and how the practices constituted a holding environment for the professionals. We also examine how different power dynamics associated in part with the multidisciplinary context seep into vulnerability practices.

Our study contributes to the literature in three ways. First, we introduce the novel concept of *vulnerability practices* to refer to the sequence of moves that group members typically enact when they display, or lead others to display, and address the doubts and insecurities experienced in the performance of their work roles. We show how a ritualized sequence of moves carried out in a bounded social setting may constitute a holding environment that encourages professionals to make themselves vulnerable and reflect on new ways of enacting work roles, despite institutional norms that inhibit this. We derive an inductive process model that explains how vulnerability practices play out, moving from an individual disclosure phase to a communal reflection phase. We show how a delicate tension between social pressure (to display vulnerabilities) and social support (to contain them) underpins vulnerability practices and participants' appraisals of them. We identify the pivotal role of theorizing practices in the transition from individual disclosure to communal reflection. The study innovates by illuminating distinctive practices that professionals can engage in to manage role-based doubts and insecurities that are very often neglected.

Second, we uncover the tensions that play out in episodes of vulnerability practices, revealing how participants' appraisals of past episodes shape their willingness or reluctance to engage in subsequent episodes, reinforcing or undermining the capacity of a setting to serve as a holding environment for them and potentially for others. In doing so, we nuance previous research that mostly emphasizes positive effects of vulnerability display by revealing its potential risks. These insights challenge conceptions of holding environments as fixed social settings, instead viewing them processually as ongoing accomplishments that are reconstituted (or not) in each episode, depending on the nature of the interactions occurring within that episode.

Third, we theorize how power dynamics shape vulnerability practices in multidisciplinary settings, arguing that power seeps imperceptibly into interactions. We show how some elements of ritual contribute to flattening power relations, while others do not, constituting environments that may "hold" some participants more than others. We also show how the variety of interpretive resources available to multidisciplinary settings can offer expanded understandings of role enactment lacking in monodisciplinary settings. The study reaches beyond prior research by revealing how multidisciplinary and power relations may constrain or enable the potential to create

holding environments allowing professionals to share vulnerabilities and learn from them.

THEORETICAL BACKGROUND

Vulnerability in Professional Work

Institutional discourses emphasize the image of professional workers as invulnerable, confident, and secure in their knowledge and expertise (Browning, Meyer, Truog & Solomon, 2007; Compagni, Cappellaro & Nigam, 2023; Pratt et al., 2006). Yet, this image may be illusory. Collinson (2003) has argued that workers across domains are afflicted by doubts and insecurities about how their role performances are perceived by others, leading to widespread status anxiety. Some have referred to the "imposter syndrome"—"a belief that one is not as capable or adequate as others think" (Knights & Clarke, 2014: 341) caused by a perceived need to display strength while hiding doubts and insecurities about role performance (see also Bothello & Roulet, 2019; Bourgoin & Harvey, 2018). Studies on executives and managers have documented angst and social discomfort associated with opening up about inability to always "get it right" (Corlett et al., 2021; Hay, 2014).

In other words, professionals often feel unable to express their vulnerabilities to others in ways that might provide them with the support and reassurance they need. These experiences have been described in health care (Delgado et al., 2020), veterinary work (Clarke & Knights, 2018), academia (Knights & Clarke, 2014), and consultancy (Bourgoin & Harvey, 2018; Gill, 2015), which all value confidence and self-control. For example, health care professionals may feel helpless in caring for complex patients, but unauthorized to share how they feel (Delgado et al., 2020). Consultants working for elite firms see themselves as falling short of role expectations, but isolated with hidden doubts and insecurities (Gill, 2015). Elite chefs feel the need to show that they can endure pain, harassment, and sleep deprivation to meet expectations (Burrow, Scott & Courpasson, 2022). Such accounts highlight professionals' unfulfilled need for validation to shore up their sense of competence. Yet, actively seeking validation might reveal such hidden vulnerabilities.

In light of this Catch-22 situation that leaves people suffering in silence for fear of their colleagues' reactions, some have suggested reconceptualizing vulnerability not as a weakness, but as a universal condition that can bind people together (Butler, 2016; Creed et al., 2022). Building on this view, scholars have cast reciprocal displays of vulnerability as a

reflexive praxis that enables learning and growth (Corlett et al., 2021; Delgado et al., 2020; Hibbert et al., 2022; Jordan, 2008). However, this raises the theoretical puzzle of how to create and maintain a social setting in which such openness is made possible despite prevailing institutional norms. As Jordan (2008: 212) indicates, it takes courage and trust to move beyond invulnerability and become part of a collective, thus transcending the “illusion of the separate self.” This leads us to consider research that focuses on different types of potential holding environments to enable and normalize vulnerability display.

Holding Environments and Vulnerability Display

The notion of holding environment was developed by Winnicott (1965) in psychoanalytic studies of mother–child relations and emotional development. It refers to how a “good enough” mother’s “holding” enables an infant to cope with disruptive stimuli, and feel valued and secure. Management scholars have used the metaphor to refer to “a social context that reduces disturbing affect and facilitates sensemaking,” highlighting “containment” and “interpretation” as defining elements (Petriglieri & Petriglieri, 2010: 50).

“Containment,” a notion developed by psychodynamic theorist Bion ([1970] 2013) and extended to organizational settings by Shapiro and Carr (1991), occurs when a group’s “holding” of its members provides them with a sense of comfort to express and work through difficult emotions (Mobasseri, Kahn & Ely, 2024; Petriglieri & Petriglieri, 2020). Containment is enacted when group members show empathy toward one another and “accept others’ thoughts and feelings without judgment” (Kahn (2001: 269). Containment fosters trust among group members that enables “interpretation,” defined as “the provision of meanings to individuals so that they can better understand their experiences” (Petriglieri & Petriglieri, 2020: 418). Interpretation occurs when group members construct shared meaning and channel their difficult emotions toward productive ends (Kahn, 2001). Through containment and interpretation, a trusted collective can hold its members and make sense together of difficult emotions, creating a shared sense of comfort in hardship.

While some have noted that organizations can serve as holding environments (Van Buskirk & McGrath, 1999), Kahn (2001) points out that the impersonal nature of work relations in large bureaucracies tends to inhibit this, suggesting the need for dedicated arrangements. Thus, some have suggested

that mentoring programs can constitute dyadic holding environments for members (Ghosh, Haynes & Kram, 2013). Others have argued that individuals in precarious roles may constitute holding environments for themselves by creating connections with friends, places, routines, and purposes that bolster confidence and enhance their sense of vitality (Petriglieri, Ashford & Wrzesniewski, 2019). However, holding environments may also be formalized as dedicated group settings apart from regular work, where people can support one another (Kahn, 2001). Yet, precisely which practices might constitute such holding environments remains unclear. To consider this issue, we reviewed social settings outside and inside the workplace that might help understand the practices implicated in the creation of holding environments.

Outside the workplace. Mutual aid, self-help, or support groups occurring outside the workplace offer insight into collective practices of dealing with shared vulnerabilities. Notably, Denzin’s (1987) classic study of Alcoholics Anonymous (AA) meetings revealed the ritualized process by which they unfold; namely, through a structured form of storytelling in which one person presents a personal story relevant to a selected theme (e.g., relapses in abstinence) and others then take turns to build on what has been said, bringing in their own personal “second stories” of the phenomenon (Arminen, 2004). Because AA meetings and other types of mutual aid groups (Borkman, 1999; Frigerio & Montali, 2016; Schrock, Holden & Reid, 2004) provide emotional support to peers experiencing similar issues (i.e., containment) and ideational resources to understand and overcome difficulties (i.e., interpretation), they can be seen as potential holding environments. Yet, there are elements of mutual aid groups that may not apply to professionals in the workplace who do not share vulnerabilities related to personal situations—and who may be more averse to potential “loss of face” with work colleagues, given the status they have to maintain.

Also outside the workplace, counseling groups are settings in which trained therapists assist individuals experiencing specific problems—for example, health, family, and life concerns (Corey, 2023; Yalom & Leszcz, 2020). Such groups meet to work toward changing problematic perspectives, emotions, or behaviors. Although counseling groups may provide containment and interpretation, their relational and process dynamics differ from sessions dedicated to professionals in the workplace. Specifically, in counseling groups, there is an emphasis on progressive behavioral change, and a clear distinction

between therapist and clients, unlike groups of professionals who seek mutual support and validation, and who are bound to each other by work relationships.

Within the workplace. Turning to social settings within the workplace, clinical debriefing sessions (conducted to discuss and make sense of critical incidents) are settings where professionals may expose their vulnerabilities. For example, Mitchell and Everly (1996) outline a seven-step “critical incident stress debriefing” methodology that asks participants to describe in order the facts of the incident, then their thoughts, then their feelings, before discussing stress symptoms and developing lessons. Professionals may view these sessions as a means to process difficult emotions and learn to cope with self-blaming (Berchtenbreiter, Innes, Watterson, Nickson & Wong, 2024: 291). However, other research reveals that junior professionals may deploy various tactics to skip sessions or resist displaying vulnerability for fear of being blamed or portrayed as incompetent by senior colleagues (Sweeney et al., 2020), reflecting failure to enact a holding environment.

In medicine, “Balint groups” are intended to help physicians reflect on clinical experiences and learn about doctor–patient relationships. Proponents (e.g., Salinsky, 2018: 9) note that they “provide a safe, confidential space where colleagues will listen to you and respect your views,” suggesting the enactment of holding environments. However, studies of these groups also reveal their potential to provoke anxiety (Clancy, Mitchell & Smart, 2020; Graham, Gask, Swift & Evans, 2009; Kjeldmand & Holmström, 2008). For example, participants in Pinder, Mckee, Sackin, Salinsky, Samuel, and Suckling’s (2006) study explained that they felt the need to hold back aspects of their experiences in group sessions.

Other social settings where people can share vulnerabilities relating to work role performances include “action learning sets”—collaborative settings where managers meet to think critically about problems experienced at work (Corlett et al., 2021; Marsick & O’Neil, 1999). These settings may enable enactment of holding environments where executives can distance themselves from restrictive role expectations and accept to “become vulnerable” (Corlett et al., 2021: 438). Yet, Corlett et al.’s (2021: 425) study also reveals ambivalence among participants who need to learn to “become comfortable with being uncomfortable.” Taken together, these studies reveal potential benefits from participation but also possible reticence to participate.

Overall, research on various kinds of social settings in which participants display or lead others to

display vulnerability reveals the importance of storytelling rituals to enable the expression of doubts and insecurities. It also shows that these settings may vary in the facilitation processes used and the nature of relationships among members, both of which can influence participants’ willingness to display vulnerability. However, we found limited fine-grained research on the processes involved in displaying and addressing vulnerabilities associated with role enactment. That some studies point to resistance to partaking in vulnerability displays highlights the potential social pressure to participate—a subtle yet important element of holding environment practices that has not been elaborated. As importantly, there is an overriding focus in the literature on vulnerability practices among individuals belonging to the same occupation or social category, while the context of multidisciplinary has been underexplored, as we review next.

Multidisciplinary as a Challenging Context for Vulnerability Display

Studies of intergroup relations in organizations (Alderfer & Smith, 1982; Mobasseri et al., 2024) suggest that inequalities associated with the presence of different categories of participants might create challenges in enacting holding environments. Yet, studies that explore vulnerability displays do not directly address multidisciplinary dynamics. What is missing in existing studies is a fine-grained understanding of the sequence of practices unfolding in multidisciplinary settings, and how interactions among people with different sources of power and epistemic assumptions may or may not enable them to reveal and work through their vulnerabilities.

Multidisciplinary settings are increasingly common as professionals with different forms of expertise need to work together. The pressures on professionals to appear confident and in control may be more intense in multidisciplinary groups, where jurisdictional negotiations influence interactions among colleagues, making them reluctant to expose their vulnerabilities. Studies of multidisciplinary work in health care have shown how lower-status professionals often hesitate to express their viewpoints (Lee, Ong & Martimianakis, 2023; Oborn & Dawson, 2010). This suggests a heightened sense of vulnerability among lower-status professionals and trainees, confirmed by studies in other fields (Ahuja, 2023; A.D. Brown & Coupland, 2005).

In other words, multidisciplinary power dynamics may complicate efforts to create and maintain a

holding environment. Indeed, Kahn (2001: 274) comments that interactions in such settings “are shaped by the boundaries, power differences, affective patterns, cognitive formations, and leadership behavior that mark relations.” Thus, a new theoretical puzzle arises as to how the power inequalities and task dependencies among members of a multidisciplinary group might shape vulnerability practices occurring in professional work settings. These considerations lead us to reiterate our research question: *How and with what perceived effects do professionals in multidisciplinary settings engage in practices that enable the sharing of doubts and insecurities experienced in the performance of their work roles?*

METHODS

Research Context

Our study was conducted in an academic primary care clinic in Quebec, Canada, that delivers care and trains residents (physicians in their last two years of practical training) in family medicine. Health care is a rich setting to study vulnerability practices as professionals frequently feel uncertain, ineffective, or helpless when faced with difficult patient cases, yet are reluctant to display their vulnerability to colleagues (Delgado et al., 2020).

The activity observed was a weekly group session described as serving three functions by the staff psychologist who created and usually facilitated it: exposing medical residents to relational practices as part of their training, “developing a team culture” among professionals working at the clinic, and maintaining a “space for reflection.” On the last function, the psychologist noted, “This is important for human work. If we do not have that, we are at risk of not caring enough, of dehumanizing care, and of dehumanizing ourselves as carers.” Facilitated by the psychologist and a teaching physician, these three-hour sessions were held online during COVID-19 lockdown. In a dedicated segment of these meetings, participants were invited to share how they felt about specific patient cases experienced as emotionally difficult, and then reflected together to learn from these experiences. Some participants mentioned the Balint tradition as an inspiration, but the presence of multiple disciplines and of “patient partners” made it somewhat different. Patient partners are themselves patients with significant experiential knowledge of illness and care, engaged as allies of professionals to connect with patients and help them pursue their life goals (Panaite, Desroches, Warren, Rouly, Castonguay & Boivin, 2024).

The weekly meeting typically assembled a senior psychologist, two psychology interns, two senior primary care physicians, two medical residents (labeled “residents” from now on), two social workers, a nurse, and one or two patient partners. The participation of residents and psychology interns was a mandatory part of their training. While other participants were regular attendees, residents rotated through the activity over time—typically, attending four consecutive meetings and then being replaced by other residents. Meetings usually brought eight to 10 participants together.

Data Collection and Analysis

We adopted an insider–outsider approach (Louis & Bartunek, 1992). The first author conducted nonparticipant observations and semi-structured individual interviews, and led initial data analysis. The fourth author—a member of both the team studied and the research team—was a participant observer and contributed to analysis and interpretation (insider researcher). The second and third authors did not participate in data collection (outsider researchers) but contributed to analysis and theorizing. Over a period of 10 months, the first author attended and recorded 20 weekly meetings lasting about three hours each and conducted 19 interviews, each lasting about 60 minutes on average (see interview guide and ethics disclosures in online Appendices 1 and 2).¹ At this point, similar themes were repeating, and observations were not leading to new insights.

Following participatory research principles (Sharma & Bansal, 2020), the first author also conducted seven group discussions with participants about emerging themes. Interviews occurred toward the end of the observation period, in parallel with group discussions. The purpose of this overlap was to infuse findings with insider understandings (Becker, 1958; Dutton & Dukerich, 2006). In group discussions, the first author introduced a general theme (e.g., power dynamics or collaboration) and then asked participants to comment on how they viewed these dynamics within the team. When facilitating these discussions, the author sought clarification from participants but did not share analytical insights. The empirical material collected represents 93 hours of audio-video recording and 960 single-spaced pages of verbatim transcriptions and observation notes. The quotes presented in the paper were translated from French by the authors.

¹ https://osf.io/95baf/?view_only=e50c5c4223dd49ec9b207037ebf2c33b

The first author led data coding and analysis using an inductive approach (Charmaz, 2014) and relying on extensive input from and discussions with coauthors. The second and third authors read meeting and interview transcripts, and watched selected video segments to participate actively in analysis and theorization. Coauthors actively challenged, improved, and validated emergent interpretations, with differences resolved by returning to the data. The coding structure is provided in online Appendix 3 with illustrative data for key codes shown in Tables 1–9 (following). The fourth (insider) author led description of the empirical context and helped in interpreting experiences of participants. All authors significantly contributed to the writing. We conducted analysis in six phases.

Phase 1: Extracting episodes of vulnerability practices. Early in our observations, we were intrigued by how participants shared their doubts and insecurities as they presented difficult patient cases for discussion with colleagues. We also noticed that these discussions seemed to play out according to patterned interactions. This led us to identify “vulnerability practices” as the focus for our study. To isolate vulnerability practices for deeper analysis, we drew on the concept of “episode,” a “sequence of communications structured in terms of its beginning and ending” enabling a collective to suspend its routine structures of communication and hierarchy to facilitate reflexive practice (Hendry and Seidl 2003: 176). Across the 20 meetings, we identified 27 distinct episodes of vulnerability practices (see Table 1). According to Hendry and Seidl (2003), episodes have their own discursive structure that stands out from familiar routines. The episodes were initiated when a person we identify as the “facilitator” invited a participant (whom we designate the “storyteller”) to “take the mic” (a term we use metaphorically) and share doubts and insecurities experienced in a difficult patient case. The episode concluded when the facilitator took back the mic and moved on to a different discussion, or signaled the end of the meeting.

Phase 2: Identifying roles within episodes. We analyzed each episode of vulnerability practices to identify the different roles played by participants, which we labeled as “facilitator,” “storyteller,” and “colleagues.” We noted that the *facilitator* was by default the senior psychologist who created the activity. A specific senior physician specializing in mental health filled in as facilitator when the senior psychologist was absent. The second role was *storyteller* (the person presenting a difficult case). We labeled all other participants as *colleagues*. Across

episodes, residents were invited more frequently by the facilitator to act as storytellers, reflecting the training mission of the clinic. Overall, the storyteller was a resident in 11 of the 27 episodes, a senior physician in seven, a patient partner in five, a social worker in three, and a nurse in one episode. Taken together, senior physicians and residents acted as storytellers in 18 episodes, while psychologists never did.

Phase 3: Coding for practices and analyzing their chronology. After identifying roles in the episodes, we engaged in open coding of the particular moves (types of utterances) enacted by the facilitator, the storyteller, and colleagues, documenting the chronological back-and-forth of interactions. By comparing and grouping the initial codes across episodes, we identified a set of discrete practices related to vulnerability display among participants (supplementary data provided in Tables 2–8, below). We noted how these practices tended to unfold according to a similar ritualized sequence for most episodes. We present the *typical sequence* of vulnerability practices (25 episodes) in the first Findings section below (illustrated in Figure 1). Through this analysis, we noted that, in two episodes, a practice was missing, or practices unfolded in an unusual sequence. We viewed these episodes as *path anomalies* that revealed important theoretical insights into the dynamics of vulnerability practices. We analyze these anomalies in the second Findings section.

Phase 4: Coding for the perceived effects of vulnerability practices. The above analysis led us to question how vulnerability practices were appraised by participants. To address this, we coded individual interviews and group discussions, identifying the perceived benefits and risks of the practices, and how these might change over time through participation in group meetings. In the third Findings section, we explain the perceived effects (Table 9, below, provides supplementary data).

Phase 5: Analyzing power dynamics related to seniority and discipline. We then examined both typical episodes and the path anomalies to identify power dynamics related to seniority and disciplinary differences. We considered who invited others, and who were invited to make themselves vulnerable, who displayed initiative within episodes, who acquiesced to or resisted others’ demands. This allowed us to document how power dynamics related to seniority and disciplinary differences shaped participants’ engagement in vulnerability practices, and how this might influence their appraisals of the activity. The codes related to power dynamics are illustrated in

TABLE 1
List and Summary of All Episodes

Episode	Storyteller	Brief summary
E1	Resident 1	Storyteller talks about substance-dependent patient described as irresponsible and unreliable. Shares feeling powerless, ineffective. Senior physician validates the approach; colleagues provide advice.
E2	Patient partner 2	Storyteller talks about patient who asked for medical assistance in dying (“MAID”). Expresses compassion for moral distress of young physicians facing requests for MAID. Senior physician and other patient partner validate and offer to help.
E3	Senior physician 1	Storyteller talks about elderly man whose wife has Alzheimer’s. Uses humor; shows attachment and compassion for patient. Exchanges among colleagues about fear of losing control due to illness.
E4	Patient partner 1	Storyteller talks about patient making excessive demands on her time and involvement. Shares feeling overwhelmed. Colleagues express compassion, empathize by sharing cases where they have faced comparable situations; advise to “place the frame.”
E5	Resident 4	Storyteller shares case of substance-dependent patient. Talks about feeling manipulated by patient, insecure about his approach, and exhausted by patient’s excessive demands. Colleagues provide validation and advice about “placing the frame.”
E6	Resident 3	Storyteller talks about woman with precancerous lesion who refuses minor intervention that would easily remove the tumor. Feels helpless about inability to convince patient to accept intervention. Colleagues empathize and theorize professional distance.
E7	Resident 3	Storyteller talks again about the case discussed in E6, expresses moral distress about patient’s consistent refusal of intervention to remove precancerous lesion. Colleagues validate feelings and advise to let go, respect patient’s pace and will.
E8	Patient partner 1	Storyteller discusses a patient seen by colleagues as somatizing. Shares her humiliating experience of having been repeatedly invalidated by caregivers and argues that colleagues should listen to the patient rather than dismissing the problem as being in her head. Colleagues express appreciation.
E9	Social worker 1	Storyteller starts by talking about feeling troubled and insecure with respect to a case. Colleagues offer directive and insistent advice but no validation. Facilitator tells storyteller three times that she should apply the “intervention model” taught to residents.
E10	Patient partner 1	Storyteller talks about excessive demands from patients who call her almost every day. Feels isolated and overwhelmed. Social worker 2 offers support as a “resource person.” Colleagues validate emotions and advise to “place the frame.”
E11	Senior physician 1	Storyteller talks about the case of young woman who has been victim of repeated sex abuses and self-mutilates. Indicates the case is “extremely sad.” Colleagues point to potential resources at and outside of the clinic and theorize self-mutilation.
E12	Social worker 1	Storyteller says that heavy workflow makes her unable to offer longer-term follow-ups needed by patients. Shares a sense of helplessness. Colleagues empathize and theorize similarities between constraints on her role and theirs.
E13	Resident 6	Storyteller talks about a woman with multiple psychosocial problems. Shares doubts and insecurity about not doing enough. Colleagues validate her approach and emotions.
E14	Resident 5	Storyteller talks about patient taking opiates for chronic pain. Shares feeling unsure about her approach. Colleagues validate her approach and emotions. Patient partner shares that she too takes opiates for chronic pain.
E15	Senior physician 2	Storyteller talks about patient described as “hypochondriac” who has excessive demands and is dissatisfied with physician’s lack of availability. Says it creates a “climate of terror.” Colleagues validate and advise to address his unease directly with the patient.
E16	Senior physician 2	Storyteller talks again about case discussed in E15, observes that the therapeutic relationship is broken. Shares feeling unsure if he should terminate the follow-up. Another senior physician validates and advises to address issue directly with patient.
E17	Resident 8	Storyteller talks about single mother on leave of absence who attributes improvements solely to antidepressants. Shares doubts and insecurity about her approach. Colleagues validate her approach and provide advice about the relationship.
E18	Senior physician 1	Storyteller talks about case of a woman with a dead fetus in utero in 2018, then a premature baby who died nine months after birth. Indicates the case is “extremely sad.” Colleagues empathize and theorize grief.
E19	Resident 7	Storyteller talks about single mother of two who has anxiety and hypertension. Colleagues question his insistence on her to see a psychologist, and ask how he feels about the case. Keeps a detached stance throughout despite being questioned repeatedly on his emotions by colleagues.

TABLE 1
(Continued)

Episode	Storyteller	Brief summary
E20	Patient partner 1	Storyteller talks about woman who was refused opioids and went to buy it on the street. Expresses outrage, advocating that physicians should trust patients and stop thinking they are all addicts. Senior physician provides medico-legal view on opioid prescriptions, offers help with the case.
E21	Resident 8	Storyteller talks about woman asking for a leave of absence due to overwork. Expresses feeling unsure about his approach. Other resident empathizes with difficulties of such cases. Colleagues discuss therapeutic value of leave of absence and offer advice.
E22	Nurse	Storyteller talks about single mother of three who is overworked. Expresses insecurity with her approach. Senior psychologist validates her approach. Colleagues theorize the importance of taking time to genuinely listen to the patient.
E23	Social worker 2	Storyteller talks about young woman with major functional handicap who—without giving consent—is being administered antipsychotics by a specialized facility. Expresses feeling troubled and helpless. Colleagues offer validation and directive advice.
E24	Senior physician 1	Storyteller talks about man described as having schizoaffective disorder who is in full-blown psychosis and does not take medication for severe diabetes as prescribed. Shares feeling powerless and insecure. Colleagues empathize and theorize treatment order.
E25	Senior physician 2	Storyteller follows both husband and wife who have a tense couple relationship. Shares feeling stuck in the middle; expresses annoyance, insecurity, and feeling overwhelmed. Colleagues theorize relational boundaries, validate emotions, and provide advice.
E26	Resident 9	Storyteller talks about patient whose wife with incurable brain cancer requires high levels of assistance. Shares feeling compassion, but also detachment and helplessness. Colleagues theorize prolonged grief, validate approach and emotions.
E27	Resident 10	Storyteller talks about woman with generalized anxiety and panic attacks. Shares doubts about his approach. Colleagues theorize the importance of building trust with patient and validate the approach.

online Appendix 4 and their role is explained at relevant junctures in the Findings below.

Phase 6: Developing a theoretical model.

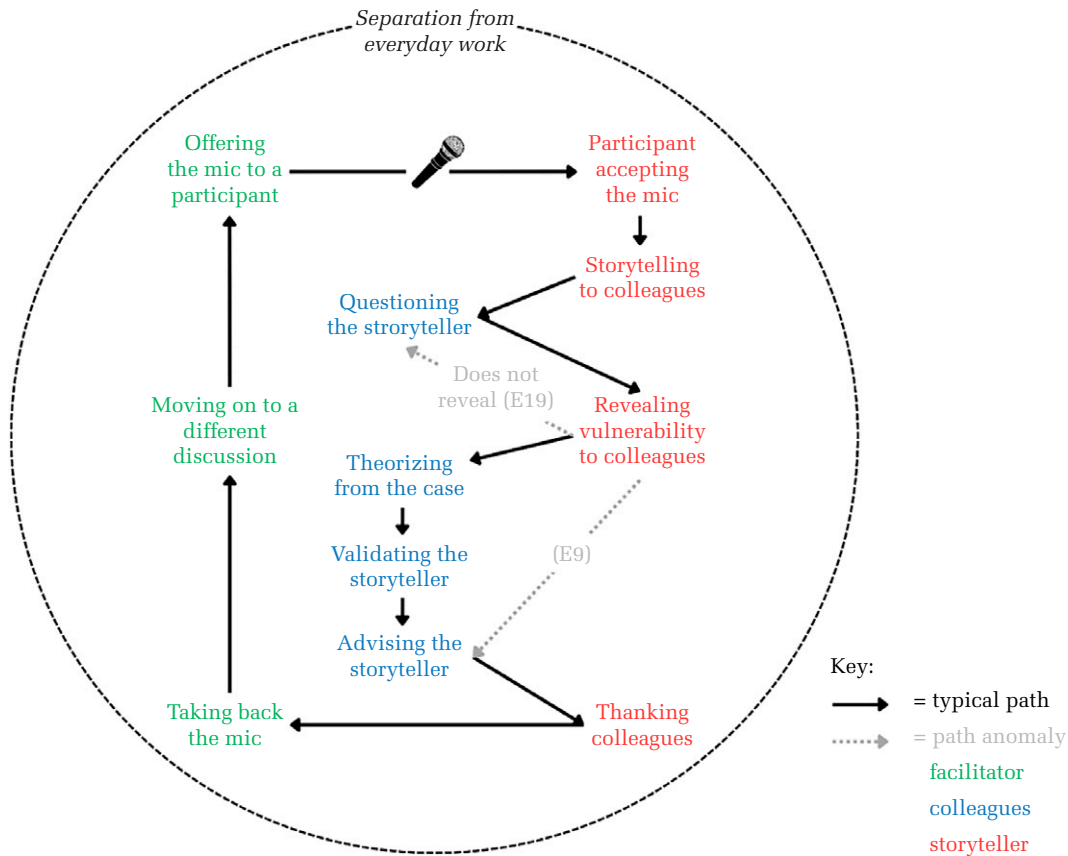
Throughout the research process, we moved back and forth between our emergent findings and relevant literature, including theory on holding environments and on ritual, as well as research on other settings where vulnerability is displayed. Building on the findings, our analysis of the typical sequence of practices suggested that they can be grouped into two broad phases (*individual disclosure* and *communal reflection*), the first characterized by social pressure to display vulnerability, and the second culminating in the provision of social support. In addition, the practice that we labeled “theorizing from the case” serves as a bridge between the two phases and can be seen as enabling the interpretive function of a holding environment. These elements form the through-line of our theoretical process model shown in Figure 2 (below). However, the observation of path anomalies, and our analyses of perceived effects and power dynamics led us to elaborate this model further to include a feedback loop accounting for participant appraisals, and to consider how power dynamics shape the tension between social pressure and social support. We unpack the theoretical model in the Discussion.

FINDINGS: A RITUALIZED SEQUENCE OF VULNERABILITY PRACTICES

Most episodes of vulnerability practices followed the typical sequence of moves illustrated in Figure 1. We begin by briefly explaining the structure of an episode before providing detailed examples. Specifically, an episode began with the facilitator *offering the mic* to a participant. After *accepting the mic*, the participant began *storytelling to colleagues*—that is, sharing a particularly difficult or unsettling patient case they faced during clinical work. Colleagues then began *questioning the storyteller* to learn more about the case and the storyteller’s experience of it. Typically, the storyteller responded by *revealing vulnerability* in different ways. At this point, colleagues began *theorizing from the case* to uncover general principles applicable across cases and disciplines. This was followed by colleagues *validating* and *advising* the storyteller. The episode came to an end with the storyteller *thanking colleagues*, and the facilitator *taking back the mic* to initiate a different discussion or conclude the meeting.

As shown later in Figure 2, we conceptualize this process as a ritualized sequence of practices playing out through sequential phases of *individual disclosure* and *communal reflection*. While, for clarity, we

FIGURE 1
Ritualized Sequence of Vulnerability Practices



map the practices sequentially and analyze them separately, they were not always clearly separate. In particular, we noted overlaps and iterations between practices within the two phases. The individual disclosure phase focuses on the storyteller, involving *social pressure* to incrementally draw out emotional aspects of the situation, leading to vulnerability displays. The communal reflection phase moves beyond the individual to the collective through the pivotal role of *theorizing* that enables participants to offer *social support* to the storyteller.

We begin by elaborating on the typical sequence of practices composing the two phases of the process. We then present the two main path anomalies. Finally, we examine how participants appraised the benefits and risks of participating in the activity. Throughout the findings, we address the ways in which power relations among members of the team might affect vulnerability practices.

Individual Disclosure Phase

Handling the mic. An episode began when the facilitator offered the mic to a participant and invited them to speak. The participant typically accepted the mic and began presenting a case. The senior psychologist acted as facilitator when she was present, and a specific senior family physician did so when she was not. In an interview, she talked about her role as facilitator:

So, I do take some leadership as it relates to facilitation. Organizing the meetings, trying to take a posture of listening, and managing the flow—I don't always achieve it ... These are tasks that need to be done, and ... I share it with [Senior physician 1].

Every participant interviewed identified the senior psychologist and this specific senior physician—both from professions highly ranked in the institutionalized hierarchy of health care occupations—as jointly in charge of facilitating the activity, including controlling the mic:

PATIENT PARTNER 1: It's [Senior psychologist] who is in charge of all this. And I would say [Senior physician 1], who has a very strong and imposing personality.

Storytelling to colleagues. When offered the mic, no participant overtly resisted the offer to speak. The storyteller then presented a case which they described as challenging or unsettling (see Table 2). This initial description was often several minutes long. In several instances, the storyteller offered a *clinical description* of the case, sticking mostly to "facts" and making frequent use of professional jargon. The following initial description of a case by Resident 3 is illustrative:

It's a woman in her thirties who is diagnosed with borderline personality disorder and has anxiety symptoms. She is treated with [antidepressant drug]. She has a cervical precancerous lesion. She is stuck in toxic relationships. She's on social assistance and struggles with her budget because she spends on drugs and tobacco.

Sometimes, the storyteller offered a *contextual description* instead, focusing on the patient's life experience and stage in their care journey, as Social worker 1 did here:

After she broke up, she consulted briefly, there was grief of not having had children ... She told me she feels guilty about asking for help. She's a nurse, and she said: "I wanted to help myself."

In a few instances, the storyteller offered a *critical description* of the case that portrayed the patient as unreliable, as unwilling to help themselves, or as creating unnecessary complications. This case description by Resident 1 is a good example:

I think a part of him wants to stop drinking. But my experience last time was, like, he'll say, "Yeah, I want to stop drinking." And then I ask him what his plan is, and he says: "I'll go home and open another bottle."

Questioning the storyteller. After initial presentation of the case, colleagues questioned the storyteller, progressively drawing out the reasons why the storyteller found the case challenging (additional quotes in Table 3). In some instances, colleagues questioned the storyteller *about the patient*, their needs, resources, and treatment history. These questions were typically framed in a nonjudgmental way. For example, Patient partner 2 questioned Senior physician 1 about the patient:

Is this man comfortable with how his wife [who has advanced Alzheimer's] is being cared for? Does he have particular concerns about how his wife is being treated?

Often, colleagues questioned the storyteller about *how they approached the case*, enacted their caregiver role, and went about the difficulties they faced at work. Such queries focused on the choices the

TABLE 2
Storytelling to Colleagues

Themes	Evidence
Clinical description	RESIDENT 4: So, it's a 40-year-old man who ... is known for a substance use disorder with relapses. And so, he had several episodes of alcoholic pancreatitis. SENIOR PHYSICIAN 1: It's a man past 70-years-old whose main problem is anxiety with a lot of somatization and hypochondria. But at the same time, he does have documented medical conditions, including a severe heart disease; he had two bypass surgeries, the last one in 2019. And a year or two ago, we found a polymyalgia rheumatica linked to his chronic back pain.
Contextual description	RESIDENT 6: She has always shown up to my appointments. But from what I understand, when she's overtaken by her emotions—she's having ups and downs, and more downs than ups—then the outside world disappears. PATIENT PARTNER 2: He doesn't have a large circle of family or friends. So, there will probably be two or three persons present [to his medically assisted death]. He has no children, no wife. So, this is a man who has been quite lonely in recent years.
Critical description	RESIDENT 1: Yesterday, he called the secretary ... panicked, like: "I'm not well ... Give me an appointment immediately, please." It was like 3 or 4 p.m. already ... I clearly didn't have time for him. So, I said: "Look, take an appointment with me, or, if it doesn't work, then call the walk-in clinic." RESIDENT 4: The first thing he told me over the phone is: "I need you to give me my social assistance, and that's it." So, it started like this. You know, I had reviewed the patient file, I had a plan ... And then I said to myself: "Well, that's gonna be it." So, I started with this impression.

TABLE 3
Questioning the Storyteller

Themes	Evidence
About the patient	NURSE to RESIDENT 1: Is he coming to see you for the leave of absence or to stop drinking? ... Do you think he really wants to stop drinking? SOCIAL WORKER 1 to RESIDENT 8: Does she have the money to pay for her psychologist? Does she still have insurance? And does she live alone, is she a single mom?
About the storyteller's approach	RESIDENT 5 (2ND YEAR) to RESIDENT 6 (1ST YEAR): At about what frequency do you see this patient? SENIOR PSYCHOLOGIST to PATIENT PARTNER 1: You're very, very close to these patients, and we're wondering what is going to happen with that. Is this going to wear you out? How do you see the end of this, a transfer?
About the storyteller's emotions	SENIOR PSYCHOLOGIST to PATIENT PARTNER 1: Did it calm down now [the insistent requests from patients]? ... Because it's like you've gone through "Do I keep at it, is that too much?" NURSE to RESIDENT 3: You feel a lot of discouragement and helplessness, right?

storyteller made, the resources they mobilized, and their relationship with the patient. These questions to a resident by Patient partner 1 illustrate this:

Did he explain to you what kind of relationship he had with his previous doctor? Or did you directly transition from one doctor to the next? Was there time for you to adapt?

Colleagues also often questioned the storyteller directly about how difficulties faced in the case made them feel, inviting the storyteller to express their doubts and insecurities. By *questioning emotions*, colleagues invited the storyteller to open up about their lived experience as caregiver involved in a difficult case. For example, the nurse followed up on comments from Resident 4 acting as storyteller who had initially made a critical description of a case:

So, in this relationship in which you feel utilized and manipulated ... how do you feel about having to continue seeing this patient? How does that make you feel?

Sometimes, a colleague would question the storyteller's emotions in a way that almost put words into their mouth, teasing out a specific type of response, as here for example:

PSYCHOLOGY INTERN 2: So, it seems you feel helpless in front of someone who is not very proactive in the changes he needs to make. It's like you have ideas about how to improve his well-being ... but he's not ready for that. Does that resonate with what you feel?

RESIDENT 4: Yes, that's it, that's exactly it.

The psychologists (senior and interns) and the nurse often sought to probe the storyteller's emotions related to the case. Participants of other

disciplines also sometimes questioned the storyteller's emotions. While residents rarely questioned participants of other disciplines or senior physicians about their emotions, they sometimes queried fellow residents, as one did here:

RESIDENT 5: Are you wondering—maybe I'm projecting here—sometimes, when I'm facing an impasse like this, I ... ask myself: "Is there something that I didn't think about, that I should be saying?" Sometimes I put pressure on myself: "Could a senior physician help this patient?" As a resident, I'm in the process of learning ... Could someone else have seen an opening somewhere that I haven't seen?

RESIDENT 6: Yes, that's a good point, that's how I felt. I felt a bit guilty, that maybe I didn't suggest the resource that could have helped her—I felt, like, "Is it because I lack experience that I can't give her what she would need?"

Questioning often came sequentially, beginning with a focus on the patient, then moving toward the storyteller's approach, and then their emotions, applying *social pressure* on the storyteller to reveal doubts and insecurities experienced in the performance of their work role.

Revealing vulnerability to colleagues. Questioning the storyteller and revealing vulnerability to colleagues formed a back-and-forth process through which the team encouraged vulnerability displays, redefining them as acceptable and desirable (additional quotes in Table 4). In most cases, the storyteller responded by gradually revealing a range of difficult emotions associated with vulnerability such as feeling unsure, helpless, overwhelmed, or mistrustful.

In several instances, the storyteller expressed *feeling unsure* that their approach to a case was adequate.

TABLE 4
Revealing Vulnerability to Colleagues

Themes	Evidence
Feeling unsure	RESIDENT 3: As a physician, I want her to get better, she's my patient. Because I feel guilty about leaving her like that. PATIENT PARTNER 1: She's telling me things she never said to anyone, even to her daughter. She's telling me: "I need to tell this, but I never have anyone to tell it to." So, I'm dealing with all this, but at the same time, I have my limits. I'm not a health professional. And I'm afraid of doing harm. Because it may uncover things that I can't deal with—I'm worried about that.
Feeling helpless	RESIDENT 1: I felt like the teacher in the class whom nobody listens to. That was my feeling... We were talking about helplessness, earlier, you know, well that's how I felt. SENIOR PHYSICIAN 1: It makes me feel helpless. But it's a helplessness that adds to another helplessness. First, it's the inability to treat his diabetes, and, second, it's the inability to have him see a nephrologist for his renal failure.
Feeling overwhelmed	SENIOR PHYSICIAN 2: It's very demanding. For a month or two, I've been seeing him on the list and I don't want to call him. It always takes 30 or 45 minutes. And it's always arguing, negotiating, and it's always in a confrontational mode ... and I don't find it easy. RESIDENT 1: I feel like everything is on my shoulders because he refuses to talk to any other professional. And so, I'm alone and forced to bear all of his problems at once. And so, yeah, I feel a bit exhausted at this point.
Feeling mistrustful	SOCIAL WORKER 2: It's unclear what's happening there. You know, how the specialized facility works so that care is provided without asking the mother to be there. There are these types of issues. RESIDENT 4: It's in the vibe. When I communicated with him, it made me feel like he was not really depressed. Maybe I got influenced by stereotypes when reading his patient file. But he gave me the impression of not being that depressed.

For example, Resident 1 revealed to colleagues that he felt unsure about the directive attitude he took with a patient, and suggested that a softer tone might have worked better:

Today, I told myself that I may have been a bit too tough on him because I really insisted that he contact these resources ... It's not really in my personality to be so confronting.

Sometimes, the storyteller expressed *feeling helpless* in a situation where they perceived that they lacked the resources, authority, or treatment options to reach a desired therapeutic outcome. For example, Social worker 2 revealed feeling helpless about his inability to establish effective communication with the care facility where a physically impaired patient resided:

I'm unable to dialogue with [workers at the facility], to know what they think ... So, if they don't return my calls, at some point, I feel compelled to call the clinical advisor and say: "Look, they're not calling me back. What's going on?"

In some instances, the storyteller shared with colleagues *feeling overwhelmed* by patients asking for more than they could provide. For example, Patient partner 1 became visibly distressed as she revealed to colleagues feeling overwhelmed by several patients

in dire circumstances who called her almost daily for emotional support:

In the last two weeks, it's been a series of calamities for some patients. I feel vulnerable because I'm unable to tell them: "No, you can't call me." ... It's so intense that I make myself available for them. I'm not sure that's good, and that's why I'm sharing that with you.

Several episodes involved a storyteller expressing that they *felt mistrustful* toward a patient or other people involved in a difficult case, doubting their motives and intentions. For example, Resident 4 revealed his doubts about a patient's sincerity toward him:

He's telling me he's super depressed, but then I look at him, and he's making jokes during the consultation. I'm having a hard time trusting him. I feel a bit manipulated.

Participants new to the activity tended to require greater pressure from colleagues during the individual disclosure phase to tease the emotional dimension out of their story. This appeared to be especially true of residents who, unlike other participants, attended only four consecutive meetings. Senior physician 2 shared in an interview that he saw himself as a role model for residents, showing them that

revealing vulnerability was authorized and expected in this setting:

The longer the residents stay, the more they realize that even us, the established professionals, we're giving the example by placing ourselves in a position of fragility by exposing our limits. It opens the door for residents to do it as well ... It's not part of the medical culture. When there's a relational difficulty, we tend to say, "There's a problem with the patient," not "I'm having a hard time dealing with these feelings."

Sharing with colleagues that one felt unsure, helpless, overwhelmed, or mistrustful in specific work contexts were ways for a storyteller to reveal vulnerability. Once this had happened, colleagues had on hand the raw material needed to theorize from the case, relating the storyteller's experience to broader principles and turning hardship into an opportunity for role-based learning; in other words, to move to the *communal reflection phase*, which we analyze next.

Communal Reflection Phase

Theorizing from the case. After leading the storyteller toward revealing vulnerability, colleagues began

theorizing from the case (see Table 5). Often guided by the facilitator, colleagues sought to derive general principles that applied across cases and disciplines to interpret what the underlying problem was, and how they might approach similar cases. At this stage, participants reflected on ways of being and feeling that authorized them to deviate from institutionalized role expectations. This interpretive effort unfolded as an interplay between the individual and the team to show how a person's particular experiences reflected a broader phenomenon encountered by all caregivers (note use of "we" and "us" in quotes below).

In some cases, colleagues reflected on *boundary setting*, by which we mean managing patients' expectations by placing explicit limits on the intensity and duration of the caregiver's involvement in the therapeutic relationship. Participants used the expression "placing the frame" to refer to this. The senior psychologist, the leading proponent of this principle, described the importance of boundary setting and how to use it with patients:

When we take the posture of caregiver, we provoke a regression on the part of the other ... This is part of what happens in therapy ... And I think it's much

TABLE 5
Theorizing from the Case

Themes	Evidence
Boundary setting	SENIOR PSYCHOLOGIST to PATIENT PARTNER 1: There's a symbolic function to the frame, which can help us think about and understand what is happening. And outside of the frame, we're a bit like in the fog ... Sometimes we need to bring back the frame [i.e., establish clear boundaries to the caregiver's relationship with the patient], to clarify and be more explicit about where we're at. PSYCHOLOGY INTERN 2 to PATIENT PARTNER 1: The frame, I was conceptualizing it as a relational device; and, when I was a kid, I always asked my mom for permissions because I knew that she always said "yes." And my mom, she knew that, in these moments, she would go see my dad... The frame, it's the third person, it's externalizing the demand [i.e., depersonalizing the relationship with the patient by setting clear boundaries to the institutional role of the professional caregiver in the context of service provision].
Categorizing	SENIOR PHYSICIAN 2 to SENIOR PHYSICIAN 1: The patient is hypochondriac, and he acknowledges it. But he also seems to be quite obsessional ... And to cope with [his anxieties], he tries to create order—which means that he calls his specialists, he searches on the Internet, he tries to accumulate knowledge. NURSE to RESIDENT 8 (<i>theorizing the typical phases that a patient will go through after a leave of absence is prescribed</i>): Patients who have an adjustment disorder related to their work, or who have burnout, there's a period where they feel guilty about not being at work. And it takes more than two weeks sometimes to stop ruminating about that, and say: "All right, now I'm resting and they can live without me at work." And the fact of having a burnout is also probably related to overinvestment at work. So you can't just unplug, rest, and re-plug. There's a fade-out period to go through, and it takes more than two weeks.
Perspective shifting	PATIENT PARTNER 1: You know, we [patients] are not all the same. Some patients don't want to act in partnership, and it's entirely fine. And there are some who need us [caregivers] to ... take them by the hand, and tell them: "Here's the treatment, here are the medications, here's this and here's that," and that makes them feel at their ease, they feel good this way. PATIENT PARTNER 1: When we receive a diagnosis, especially of cancer, that word has an effect. Having heard it three times for myself, I can tell you that it has a devastating effect ... It's about the way we say it ... There's not a single family in the world that is protected, that did not have or will not have someone who has cancer. So, this cancer, it's the monster. And receiving the diagnostic, it's absolutely devastating. One never gets used to it.

easier to deal with that when we have a frame: “What I’m offering you is one meeting per week, perhaps two.”

Boundary setting was a theorizing practice that sometimes led colleagues to reflect on the role of caregivers in society. For instance, Senior physician 1 shared these thoughts about the boundless expectations that patients sometimes entertain about the physician’s role and capability:

Nothing stops a family physician from prescribing antipsychotics, from doing injections or X-rays ... from transgressing his field of expertise. Sometimes, it is an invitation to take on an omnipotent role ... It’s the patient’s absolute desire to be well, to be cured, and quite often we get carried away by that. Our desire to heal, to do good by any means possible. And sometimes it creates huge tensions ... Things go well as long as we heal scratches. But then, you can also become the village chief and call all the shots.

Ideas about “letting go” of the pressure of being an ideal caregiver, taking the time needed to establish trust, and genuinely listening to what the patient has to say, were also related to boundary setting. Letting go was a way of managing the call to omnipotence that physicians, and especially residents, tended to struggle with. This exchange illustrates efforts to set boundaries:

PATIENT PARTNER 1: And the ideal caregiver, it doesn’t exist, we know that. But having a heart, being there for us [the patients], it’s already a huge thing.

RESIDENT 5 (2ND YEAR): Yes, thank you, thank you. Sometimes we need to be reassured. There are patients’ expectations, our own expectations about ourselves, the expectations of our supervisors ... It helped me a lot to talk about it and hear your views.

RESIDENT 6 (1ST YEAR): When you brought up the situation, your thinking was: “What did I do wrong? Was I within the frame?” ... It seemed like you felt a lot of guilt, and I understand you so much because I feel that a lot. We’re under such performance pressure, we’re learners, and, when it doesn’t go as we would like, we tend to blame ourselves.

In several episodes, colleagues theorized about patients’ behaviors and attitudes by *categorizing*, implying that difficulties encountered were attributable to a known condition—described by a clinical label or category assigned to the patient—rather than to the caregiver having adopted a misguided approach. This theorizing practice alleviated the storyteller’s doubts and insecurities by attributing the responsibility for difficulties to the patient’s shortcomings rather

than the caregiver’s incompetence. The nurse, for instance, commented:

Patients with personality disorders put us in metaphorical danger. We feel powerless, persecuted. They often push on our weaknesses, they’re very intuitive, we feel threatened. So, it’s very important to listen to that, as a professional, this sense of danger. So, striving to support each other in this [caregiver and patient] relationship is important.

Theorizing also often involved *perspective shifting*. The patient partners, who were the only participants in the activity who authorized themselves to speak from a patient perspective, were typically the ones engaging in this theorizing practice. They did so by shifting the focus toward the patients’ lived experience of care. This might lead colleagues to consider that patients did not necessarily expect caregivers to provide solutions to all their problems, but that simply listening without judgment and with genuine empathy could be therapeutic. For example, later in the exchange reported above involving Residents 5 and 6, Patient partner 1 added:

The patient’s number one thing is to be listened to, and the doctor’s office is a safe space ... What we want so much is to be ... valued in our humanity rather than being seen as a body, an illness ... When we suffer, and when we are on opioids, we hope to find someone like you [Resident 6] who understands us without judging, without thinking that we are drug addicts.

Note that the last two theorizing practices both focus on the patient, but from different perspectives. Categorizing took an external view, explaining difficulties experienced by caregivers as caused by the patient. In contrast, perspective shifting led participants to consider the care relationship from the patients’ experiential perspective, focusing on what was important to them.

Overall, theorizing from the case was an interpretive process over which some participants (e.g., senior psychologist, patient partner) exerted greater influence than others because of their distinctive expertise. Theorizing allowed colleagues to derive general principles from a case and to reflect on alternative ways of enacting work roles. It also allowed caregivers to see that difficulties they experienced were faced by others too. As Resident 5 observed during an episode:

In spaces like here, we can discuss with supervisors and colleagues, and we realize that these are things that we all experience. And what [Senior psychologist] said touched me: “Being the magical hand that solves everything is not helpful.”

The shared meanings constructed through theorizing thus allowed colleagues to offer social support by validating and advising the storyteller, as we see next.

Validating the storyteller. Based on how they theorized from the case, colleagues offered social support to the storyteller in different ways (additional quotes in Table 6). Colleagues often *validated the storyteller's views and approach* by approving how the storyteller dealt with the patient case. By doing so, colleagues sought to alleviate the storyteller's doubts and insecurities about how they performed their work role. The facilitator, for example, validated the views and the approach of Resident 1, who expressed feeling unsure, by saying:

I think you know the resources quite well already, and you referred him to many places that are entirely adequate to help him with his problems.

While senior participants often validated their junior colleagues' views and approach, such validations sometimes also came from peers showing solidarity toward each other. For example, when Resident 6 shared her efforts to put less pressure on herself and respect the patient's pace, Resident 5 validated her approach: "What you're saying is very important, I admire you for doing that." Patient partners also often offered validation, but, unlike others, they typically did so from the perspective of patients' lived experience of care, as Patient partner 1 did here:

In my view, you're doing an excellent job curing her ailments, but, more than that, you're really listening to her, which is rare ... And that's quite good.

When a storyteller made themselves vulnerable, colleagues also typically *validated the storyteller's emotions* by saying that what they felt was normal and appropriate. Colleagues frequently validated the storyteller's emotions by empathizing—that is, acknowledging that they too had experienced cases where they felt like that. Empathizing allowed participants to collectivize their experiences of vulnerability and turn them into a shared experience. For example, Resident 2 empathized with Resident 1's feelings of helplessness:

Me too, I'm having a hard time dealing with substance abuse disorders because part of the responsibility belongs to the patient. Even if we try to guide them, they need to be motivated. So, the feeling of helplessness is very normal ... I find them the most difficult of patients.

Advising the storyteller. While or after validating, colleagues often advised the storyteller to approach a case in a particular way by either *directing* them or *suggesting and exemplifying* strategies (additional quotes in Table 7). Advising with a directive tone was usually done by a senior colleague toward a junior storyteller. But a directive tone was also sometimes taken by a patient partner to advise a professional colleague in a way that we interpret as advocating for the patient. In such instances, the advice was based on the patient partner's experiential expertise of living through care situations. Below is an example of Patient partner 1 advising a resident:

So, trust her ... and stop treating us [patients] with kid gloves ... You have your responsibility, but we have ours too. When you're lucky enough to have a patient with whom you can discuss, be listened to, and listen to her as well, that's all you can ask for.

TABLE 6
Validating the Storyteller

Themes	Evidence
Validating views and approach	SOCIAL WORKER 1 to RESIDENT 6: To me, when you ask this question, it shows that you know your patient well... It shows that you have a good therapeutic relationship with her. SENIOR PHYSICIAN 1 to RESIDENT 1: You have now given yourself a role that makes much more sense ... You have defined the relationship with the patient in a much better way. And your job as a physician with patients who have substance abuse and mental health problems will always be to arrange a relationship that can serve them. So, I think there's good improvement here. I think you did an excellent job.
Validating emotions and empathizing	SENIOR PSYCHOLOGIST to PATIENT PARTNER 1: When we see a lot of suffering, we want to do what we can because it's hard to tolerate that the person is suffering so much. What I hear is that you can offer a greater presence than other caregivers. But I hear that it's a heavy burden for you. SENIOR PHYSICIAN 1 validates SENIOR PHYSICIAN 2 (<i>by revealing that that he too has been struggling emotionally with similar issues</i>): For me, it's been a major challenge in my clinical practice, in recent years, to feel at peace when I'm doing medicine and dealing with obsessional patients like this one. This climate of terror often comes from the combination of my own worries about being perfect and making sure that I don't miss anything, and the patient who does something like that as well to cope with his anxiety.

TABLE 7
Advising the Storyteller

Themes	Evidence
Directing	SENIOR PHYSICIAN 1 to RESIDENT 1: I would give him this message that no one else can quit drinking for him... Then, regarding the leave of absence—it's always the notion of incapacitation—is it therapeutic? If he drinks 10 bottles of vodka instead of two, it's not therapeutic... And then, is he incapacitated? This is the first criterion [to prescribe a leave of absence]. SENIOR PSYCHOLOGIST to SOCIAL WORKER 2: So, I think you need to put an end to this speculation [about whether there is informed consent for the psychiatric medication being administered] by getting information from someone who's responsible, and who's in charge of the care that's being offered there.
Suggesting and exemplifying	PSYCHOLOGY INTERN 1 to RESIDENT 4: There's an author in psychology who ... talks about being good enough. You know, not perfect, not ideal, not the one who will stop his drinking, and not the one doing too much either. Because I hear that, in the past, you tried to do a lot, to be very present, and it didn't work out that well. So now, just doing a bit less, being a bit stricter ... it's just good enough. PSYCHOLOGY INTERN 2 to PATIENT PARTNER 1: It's more with kids, but ... often, what I do is I suggest things to the kid by talking only about myself: "I asked myself, I wondered, etc." ... I just think out loud. And the person responds or not. ... It could be a way to approach this.

In most instances, colleagues used a suggestive tone that gave the storyteller some leeway to accept or not the advice. A frequent way for colleagues to advise was by exemplifying or sharing a situation where they had applied the advice given to the storyteller. For example, Senior physician 1 exemplified to a resident the use of humor to set boundaries on a patient's excessive demands:

It reminds me of a patient who comes to see me to be reassured—he's super hypochondriac. So, at one point, I said: "Hey, I found the solution to your problem—I'll become your roommate ... Every time you're worried about your health, you can knock on my door, I'll examine you, and you'll be reassured." So, he said: "Doc, that would be fantastic." We both knew that was unrealistic ... but it reminded him of the limits of our relationship.

Thanking colleagues. In response to validation and advice received from colleagues, the storyteller

thanked them, often expressing being reassured and relieved by the exchange (additional quotes in Table 8), as in this quote from Patient partner 1 after receiving validation and advice:

I always say that, even with goodwill, one can do harm. So, that's why I needed to talk to you. And so, I thank you very much ... Thanks for hosting me and for listening to me. I think I'll be sleeping much better tonight.

Most episodes concluded with the storyteller more or less emphatically thanking colleagues, acknowledging validation and advice, and reciprocating support received. Even when one-on-one interviews revealed that the storyteller was unhappy with how the episode unfolded, they would nonetheless thank colleagues, suggesting that there was pressure not to embarrass colleagues in ways that might undermine the legitimacy of the activity or slight its participants.

TABLE 8
Thanking Colleagues

Themes	Evidence
Expressing gratitude	RESIDENT 4 to COLLEAGUES: I really want to say thanks for all your advice. I really feel like it gives me more tools to talk with him. Honestly, thank you. PATIENT PARTNER 1: Thank you. Without knowing it, I was already doing that. And we see that the person makes progress. Here, there's something that's happening because we're talking about ourselves. And I like it a lot, thank you, really.
Expressing feeling reassured	RESIDENT 5 to COLLEAGUES: It's helped me a lot. It's reassured me in my position, that what I did wasn't wrong. And that I just need to learn to go at the patient's pace, and to continue doing what I'm doing, which is more therapeutic than I thought. SENIOR PHYSICIAN 2 to COLLEAGUES: That's plenty enough [referring to advice received from colleagues]. And that's a huge contribution in a very short period of time that you're bringing to me. And this will really help me move forward.

Overall, the sequence of vulnerability practices in a typical episode reflects the progressive unfolding and working out of the underlying tension (“Catch-22”) that motivated our research question. Professionals must express their vulnerabilities to receive social support that can assuage their doubts and insecurities, but they are often reluctant to do so because revealing vulnerabilities might reflect poorly on their competence. During the individual disclosure phase, we see how more or less social pressure (offering the mic, questioning) enabled vulnerabilities to be disclosed. Only then could participants work collectively to interpret and learn from the experiences through theorizing practices, and offer social support (validation and advice) to the storyteller. In most cases observed, the process seemed to work as described, with vulnerabilities revealed and social support offered by the group. However, occasionally, something went wrong, as we now explain.

Path Anomalies

Our comparative analysis of the chronologies of episodes led us to identify two notable “path anomalies” (i.e., two episodes that meaningfully diverged from the typical sequence of practices): one episode we call *under-vulnerability* and the other episode we call *under-validation*. We summarize these here and consider their analytical significance.

Under-vulnerability (Episode 19). When questioned by colleagues, Resident 7, acting as storyteller, consistently refrained from displaying vulnerability and returned the focus to the patient, away from his own emotions. This created a feedback loop wherein colleagues kept questioning the storyteller, who continued to refrain from displaying vulnerability. This went on until the facilitator intervened to forcibly theorize doubts and insecurities about role performance—or “anxiety,” as she phrased it—suggesting that the storyteller was experiencing such anxiety:

To me, this relates to the role of the physician ... And I'm under the impression that the theme of anxiety [discussed in the case] is contagious. You know, we can become very anxious [as caregivers] as well, like: “Am I doing the right thing? Am I doing enough?”

This led the storyteller to reluctantly acknowledge feeling insecure (“Yeah, that’s it”), allowing the sequence of practices to resume. A bit later in the episode, this exchange ensued:

FACILITATOR (*appearing annoyed*): Do you feel that our discussion had any use for you?

STORYTELLER: Uh ... yeah, in fact, it reassures me.

FACILITATOR: Ok, it reduces the anxiety (*laughing*), that’s good.

STORYTELLER: I was not really anxious for her [the patient]. In fact, I know that she does what she has to do ... I prescribed medication, I did the follow-up. We could increase [the dosage] ... It was my plan to do so, but I wanted to begin slowly with the medication.

FACILITATOR: Ok, well, uh, maybe.

In this episode, the facilitator’s repeated attempts to lead the storyteller to reveal vulnerability were met with the storyteller emphasizing technical aspects of the case. This illustrates the posture of a learner trying to abide by competing expectations—acting as both a knowledgeable practitioner (as he was trained elsewhere) and a compliant participant in the activity by agreeing somewhat with the facilitator (reluctantly indicating that the discussion had reassured him). After receiving brief validation and directive advice, the storyteller laconically thanked colleagues: “Yes, those are good points ... Thank you, indeed.” The facilitator then took back the mic, ending the episode. Unlike other episodes, here, the storyteller repeatedly resisted revealing vulnerability, which appeared to disrupt the typical sequence of practices.

This instance sheds light on the authority of the facilitator, who seeks to ensure that the activity unfolds according to the expected sequence of practices, and to the social pressure applied by seasoned participants on the storyteller to enact the expected script. This may also signal that those who played storyteller in the past expect the person now acting as storyteller to reciprocate by revealing vulnerability as well. We interpret this episode as an instance of “under-vulnerability” revealed principally in the individual disclosure phase of the process.

Under-validation (Episode 9). In this episode, Social worker 1 volunteered to act as storyteller. From the first sentence of her story, she made herself vulnerable to colleagues (“It’s a situation that has shaken me quite a bit”). She shared feeling overwhelmed with multiple complex problems faced by the patient (including domestic violence and an eating disorder) and feeling unsure about how to approach the case. Her presentation was infused with helplessness about the situation and compassion for the patient. After the storyteller’s emotional presentation of the case, Senior physician 2 asked: “What do you think is the biggest driver of the

patient's suffering right now?," to which the storyteller responded: "She tells me it's a mix of everything [emphasizing the overwhelming complexity of the case]." Colleagues then inquired about the patient's access to resources and medical diagnosis. The facilitator intervened, indicating that Social worker 1 should apply a standardized evaluation model that senior team members had designed and promoted, then addressed the storyteller: "What are her [the patient's] expectations? I hear the questions you are asking, but don't hear what the patient wants."

Having cast doubt on the storyteller's ability to identify the patient's needs, the facilitator skipped the practices of theorizing and validating to formulate directive advice and indicate some options that would take the patient out of the care of Social worker 1:

Sometimes we must think about what to do in a crisis ... The physician could see her, or we could see her, or she could be seen in the walk-in clinic. Or you could see her until other options become available.

Colleagues concurred with the facilitator's advice. During the episode, the facilitator and colleagues provided no validation of the storyteller's emotions. Nonetheless, the storyteller concluded by briefly thanking colleagues: "Thanks, it makes sense. It feels good to be reminded of it. What I did was not nothing." The facilitator responded with brief validation: "You did a lot." This instance, like the previous path anomaly, signals social pressure on the storyteller to thank colleagues regardless of whether or not they appreciated the experience. Importantly, it illustrates a lack of social support for someone who had not hesitated to make themselves vulnerable.

Skipping the practices of theorizing and validating made the advice formulated by the facilitator and colleagues appear judgmental and critical of the storyteller rather than a statement of general principles that all can learn from, as usually formulated. In a one-on-one interview a few months later, the storyteller interpreted the episode as undermining her professional competence. Specifically, she expressed having felt "unrecognized" and said that "it's not necessarily because we don't have expertise in [a specific area] that we know nothing." However, none of this was verbalized during the episode observed. This episode contributed to the detachment of that professional from the team and made her reluctant to participate in subsequent meetings. In the interview, Social worker 1 also reported feeling intimidated by the authority structure of the

activity, and mentioned that some colleagues shared her sentiment:

I think it's because of the academic structure or the hierarchy that sometimes, it can be difficult. When there are new participants, how is it going to be perceived? There are performance pressures. I was listening to a resident who said: "You know, me neither [I don't feel at ease]." I have heard others who didn't feel at ease in this team. And it's run by a psychologist and a physician ... Or maybe it's how they talk, and maybe they think it includes everyone. And they don't notice it.

Indeed, another participant in Episode 9 told us in interview that the storyteller "was treated as a trainee" and not the qualified professional she was, which had undercut that participant's willingness to talk in the meetings. We interpret this episode as an instance of *under-validation*. While the first anomaly relates to deviance in the first phase of vulnerability practices (individual disclosure), this episode relates to deviance in the second phase (communal reflection).

Participants' Appraisals of the Effects of Vulnerability Practices

Having analyzed the sequence of practices that structure typical episodes, and how "path anomalies" diverged from this pattern, we now focus on how vulnerability practices were appraised by participants as expressed in interviews and group discussions. Overall, while most participants valued the experience, for some, certain aspects of the practices were experienced as uncomfortable, or even intimidating. We first describe the benefits that participants associated with vulnerability practices, and then consider their perceived risks (see also Table 9).

Perceived benefits. The benefits of vulnerability practices as perceived by participants included receiving *comfort* (feeling included in the group and relieved from doubts or insecurities), *normalization and reassurance* (achieving a stronger sense of confidence in one's own skills as caregiver), and *role-based learning* (experiencing changes in how one viewed one's role). These were often intertwined, as the quotes in this section illustrate.

Storytellers received *comfort* from colleagues through a process described by participants as "creating a sense of belonging." For example, the nurse mentioned, "There's something that feels good in these meetings. We're all considerate toward each other." Resident 10 reflected:

TABLE 9
Participants' Appraisals of Vulnerability Practices

Themes	Evidence
Comfort	PATIENT PARTNER 1 to COLLEAGUES, GROUP DISCUSSION: When we're ... supported by a team, we feel much less isolated. I feel very isolated in what I do, it's one of my problems... And because of that, the [weekly meeting], it's vital for me to be here. Because the isolation disappears. I feel I'm part of a team. SOCIAL WORKER 1, GROUP DISCUSSION: [Participating in this meeting] allows me to see that we're not alone in this. Sometimes I think I'm the only one to face a certain kind of problem. And being able to talk about it and see that we're not alone to experience this ... The fact that here, we allow ourselves to open up about it, to talk about our vulnerabilities, I find it interesting that we feel safe enough to talk about it.
Normalization and reassurance	RESIDENT 10 to COLLEAGUES, GROUP DISCUSSION: There's a case that we've discussed twice. It's helped me a lot in my approach, in my conduct ... I think it's also reassured me about what I was doing, overall, on my ways of being and of doing things. NURSE, INDIVIDUAL INTERVIEW: I think this is a setting for us to think together and try to overcome relational dead-ends ... it gives us access to new standpoints. It helps to meta-reflect on problematic situations, and it gives us support because it validates: "Yes, you are right to find this difficult." ... We can take some distance... Here, we're in a safe and structured setting. And we get out of it better equipped. It's like: "Yeah, I didn't see it like that. I'll try this and let you know next week how it worked."
Role-based learning	SENIOR PHYSICIAN 1, INTERVIEW: I've learned a lot from [the weekly meeting]. For example, the necessity in psychotherapy to always reflect on what we're doing. The necessity of placing a frame, of having a ritual, of always doing a bit the same thing. Applying it on a daily basis gave me a structure. And, as a result, my practice is very different from that of another family doctor. SOCIAL WORKER 1, INTERVIEW: When you're feeling uneasy with the person [i.e., the patient], we've discussed that. And sometimes it can help resolve or address situations ... It can [also] help a lot to understand and educate everyone about each other's different roles [as caregivers].
Social discomfort	PATIENT PARTNER 1, INTERVIEW: My role [in the activity], I don't really understand it (<i>laughs</i>). The only thing I know is that I'm taking my place and I like it ... I'm not someone who's very self-confident. I'm very hesitant. I'm very timid, even if it doesn't show. Very insecure. But when I believe in something, when something is important for me, I'm able to take my place. RESIDENT 9, INTERVIEW: Well, I have to say that at that time [the first meetings the resident attended], I felt I was a bit more passive. I was listening more than I was talking ... But I still tried to find themes to discuss. But my role was more passive then.
Intimidation	SENIOR PHYSICIAN 2, INTERVIEW: Of course, young residents may feel intimidated by the "bosses" [senior physicians acting as supervisors] ... This is why I say that we need to familiarize them [socialize residents into how the activity works and what is expected of them]... Especially with the young residents. PATIENT PARTNER 1, INTERVIEW: What intimidates me is not the titles or the diplomas. I feel intimidated when I see someone who is so intellectually brilliant, with the knowledge and all that. I'll be attracted to these people, but at the same time I'll tend to stand back. I'll be intimidated, but in a good way.

As health care professionals ... we can experience stress and feel helpless in complex situations. These meetings allow us to relieve, evacuate some of that stress with the support of the different professionals, and to feel ... supported in our decisions and receive advice.

Senior physician 2 talked about his own experience of vulnerability and said that support from the group significantly contributed to alleviating doubts he had about his professional career:

Vulnerability is not a trivial thing in the health care world ... This case [of an extremely challenging relationship with a patient] made me question myself about quitting my job after many years of medical practice ... It was at that level of suffering. And the impact of the group in that regard was major.

Participants expressed how validation helped to *normalize* their experience and *reassure* them about their role performance, making the advice received feel nonjudgmental, and thereby soothing their insecurities and nurturing their self-confidence, as Patient partner 1 expresses here:

It gives me a sense of security to know that what I've been doing for a long time without having been trained for it, to find that, at the end of the day, it's not that bad. So, it validates ... it reassures me, it comforts me as well.

Participants also indicated that vulnerability practices shaped how they performed their professional roles (*role-based learning*). Social worker 1 expressed feeling reassured to see that colleagues across disciplines faced similar struggles: "It's not because they're

physicians or psychologists, or whatever, that they don't have difficulties with some patients. It facilitates collaboration between us." Resident 10 noted that the activity allowed him to attribute value to his role of providing support to patients for whom tangible treatments were elusive: "So, telling my colleague: 'Look, you can also just take a role of support for the patient.' It's a piece of advice that I can give now, and that I've drawn from these meetings."

In several episodes, senior physicians and residents exchanged critically about the "call to omnipotence" made by patients who expect their physician to solve all their problems, and the pressure that physicians put on themselves to heal their patients. For example, Senior physician 2 noted that the activity allowed participants to "learn that it's fine to do nothing in a situation that looks like a dead-end." He added, "There are many exchanges about the posture of care, ... taming helplessness, being reassured that, at some point, it's fine, even though you're under the impression that there's nothing to do." A similar idea was expressed by Resident 9:

Maybe just listening to our patients, being there for them when they need it, without necessarily making clinical interventions ... It can just be helping in the sense of supporting ... This is something I've learned in the meetings and integrated into my practice.

In other words, participants indicated that engaging in vulnerability practices within this dedicated setting gave them the opportunity to be comforted by colleagues, relieve feelings of insecurity and helplessness, and feel validated in their skills. In most cases, this allowed them to construct alternative role expectations that valued mutual displays of vulnerability in contrast to institutionalized norms that emphasize confidence, rationality, and self-control.

Perceived risks. The risks participants associated with engagement in vulnerability practices were related to the *social discomfort* of having to reveal their doubts and insecurities at work in a setting where they might be judged by others. Indeed, the two path anomalies we described earlier suggest that revealing vulnerability was not easy for everyone. Social discomfort was reported more acutely by lower-status participants (residents, social workers, nurses, and patient partners), and less by senior participants (senior psychologist and senior physicians) or by those who were not expected to act as storytellers (psychology residents).

For example, Senior physician 1 expressed a high degree of comfort due to his dominant status at the clinic: "I feel fully free [in this setting]. It's because I enjoy a very comfortable hierarchical position."

In contrast, some participants reported feeling overly pressured to engage in vulnerability practices and unsure what was expected of them when they first attended. Resident 10—noting the limited time frame (four weeks) over which residents participated in the meetings—said in interview: "I'm, like—I need time to adapt, but, in four meetings, I need to adapt fast ... The other participants ... are more used to speaking with each other."

This suggests that participants may feel more or less comfortable depending on seniority, discipline, and frequency of participation. The conversation with Resident 10 continued:

INTERVIEWER: Did you feel comfortable to express yourself and take part in the exchanges?

RESIDENT: Mmm ... (*hesitation*). Honestly, in the first meeting, maybe not. Now I understand how it works ... If I say something, it has to be relevant, because I should be careful about the words I use—we have patient partners with us. And, as meetings went by, I began feeling more and more at ease and bringing cases from my practice.

Interestingly, in this quote, the resident reports having felt uncomfortable because of the presence of patient partners, given that health care professionals usually exchange about clinical cases in settings where patients are absent. This remark hints at how the unusual inclusion of participants whose perspectives were based on their experiential expertise as patients may have led the professionals to question assumptions about their work roles that they often take for granted.

The "under-validation" episode shows a different problem. The storyteller here *did* make herself vulnerable, but, as we saw earlier, interpreted the absence of theorizing and validating as a negative assessment of her competence, which accentuated her insecurities rather than alleviating them, making her feel looked down upon and patronized by colleagues. Thus, while positive experiences with the team might enable participants to feel more comfortable over time, negative experiences could do the reverse, increasing feelings of social discomfort and intimidation.

Indeed, the episode illustrates how participants of lower disciplinary status—social workers, patient partners, and nurses—tended to feel *intimidated* by the knowledge and expertise displayed by physicians and psychologists, as Social worker 1 explains:

It can feel imposing. ... They have titles and all that, and I'm being asked, for instance: "Do you have a doctoral degree in social work?" "Uh, no, a bachelor's."

It's small things like this that made me feel like I had to prove myself more than they did, or do in-depth studies of things, because sometimes they're asking me questions that are so specific.

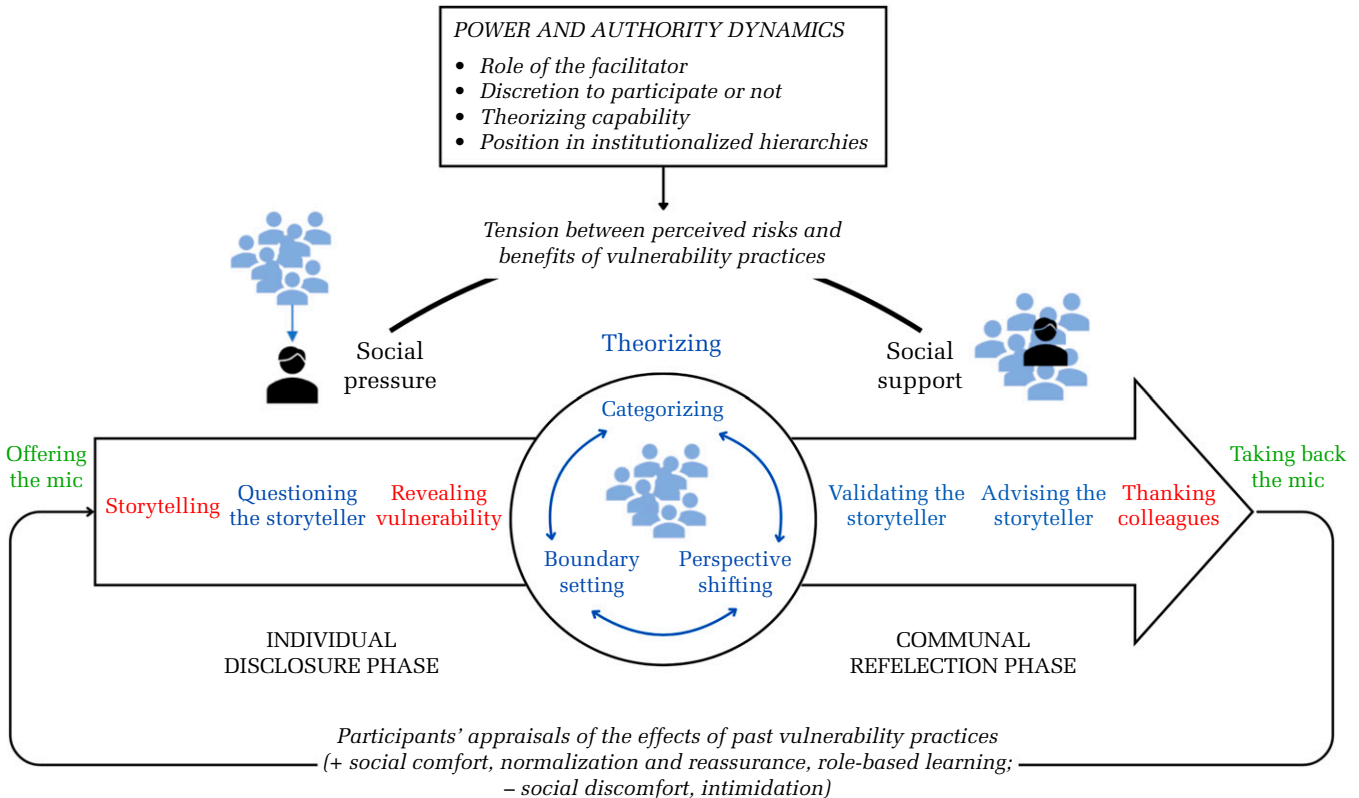
The nurse expressed, in an interview, a similar sentiment of initially feeling uneasy and unsure of what was expected of her, but gaining some familiarity over time with how the activity would unfold and how to properly play her role in it. She explains that this was made possible by realizing that other participants also felt uneasy, coming over time to experience this uneasiness as constructive and intellectually stimulating. She remained, however, reticent to speak:

I first came to this activity without really understanding what I was supposed to do there ... And, over time, I gained some confidence ... I always felt a lot of welcoming and compassion for my lack of experience [with mental health cases]. ... I feel that I'm learning, and I feel very lucky to be in contact with all these experiences. What makes me feel a bit more at ease is that there's always new residents, so that we

share some of that uneasiness ... Sometimes I want to speak, but I'm, like, "It doesn't make sense" ... But I like this feeling of uneasiness ... It makes me grow and I appreciate it. So, it's safe but not always comfortable.

Overall, engaging in vulnerability practices could be first experienced as intimidating—and could remain uncomfortable over time. As we see with the last two examples, the tension between benefits and risks was felt even by those whose overall appraisal was positive. This ongoing tension was presented as stimulating by some, for whom it came with opportunities for learning and growth, but was less so for others. The analysis of path anomalies also sheds light on the nuances of a setting in which power dynamics were shaped by disciplinary and seniority status inequalities. Vulnerability practices could be experienced as uncomfortable and occasionally, as harmful when the social pressure to display vulnerability was not balanced with corresponding social support.

FIGURE 2
Theoretical Model of Vulnerability Practices in Multidisciplinary Settings



Key: storyteller, colleagues, facilitator

DISCUSSION

In this study, we asked: *How and with what perceived effects do professionals in multidisciplinary settings engage in practices that enable the sharing of doubts and insecurities experienced in the performance of their work roles?* Our study showed how the collective enactment of a ritualized sequence of *vulnerability practices* might enable participants to make themselves vulnerable and to receive the support needed to assuage the doubts and insecurities experienced in the course of their professional work. The process model illustrated in Figure 2 builds on our findings to capture the dynamics and conditions through which participants can come to appraise these practices positively and sustain them over time. We argue that, to the extent that these practices can be sustained, they may constitute a “holding environment” (Kahn, 2001) that provides “containment” (the ability to manage threatening emotions) and “interpretation” (ideas about how to understand these difficult experiences).

Yet, we also argue that these practices can be fragile, notably when there is an imbalance between the social pressure experienced in the individual disclosure phase and the social support offered in the communal reflection phase. Building on this idea, we offer a novel analysis of holding environments at work not as fixed spaces that people enter, but as relational settings nourished and undermined on an ongoing basis by the ways in which participants enact vulnerability practices. We also add valuable nuance to research that has mostly covered the positive effects of vulnerability display at work, by revealing the perceived risks and power dynamics that pervade vulnerability practices among professionals in multidisciplinary settings. We unpack the process model and these theoretical contributions below.

Vulnerability Practices in Professional Work: Between Social Pressure and Social Support

We defined “vulnerability practices” as the sequence of moves that members of a group typically enact when they display, or lead others to display, and address the doubts and insecurities that they experience in the performance of their work roles. As shown in Figure 2, we conceptualize this process as a ritualized sequence of practices playing out through sequential phases of *individual disclosure* and *communal reflection*, with *theorizing* enabling the transition from the first to the second phase. The model shows how a delicate tension between *social pressure* (to display vulnerabilities) and *social*

support (to contain them) underpins vulnerability practices and participants’ appraisals of them. As we noted, this tension is directly related to the Catch-22 situation in which professionals find themselves. On the one hand, professionals seek social support to “contain” their vulnerabilities, but they can only obtain this by revealing these vulnerabilities, which is in part what makes them feel vulnerable in the first place because their competence and actions might be called into question. How can the vulnerability practices observed overcome this difficulty? We theorize that three aspects of the processes observed contribute to explaining this.

Ritualization of vulnerability practices. First, we argue that the *ritualized* nature of vulnerability practices can help participants to engage with them. Indeed, the set of moves we observed displays three key characteristics identified by scholars of rituals (Bell, 1992; Johnson, Prashantham, Floyd & Bourque, 2010; Lepisto, 2022; Smith & Stewart, 2011): the activity is removed from regular work, facilitated by a recognized leader, and structured by a predictable sequence. In their study of strategy workshops, Johnson et al. (2010) suggest that these features allow for the temporary relaxation of institutionalized norms, enabling participants to engage differently with each other. They note the importance of “liturgy” (the prescribed form and sequence of a ritual), and facilitation by a “ritual specialist” (equivalent to the priest in a religious ceremony) who leads and legitimizes the ritual. Liturgy generates predictability, which alleviates uncertainty (Bell, 1992; Smith & Stewart, 2011). Indeed, ritualization provides participants with advance knowledge of what is likely to happen during an episode (e.g., revealing vulnerability normally leads to comfort, reassurance, and learning) as well as clarifying what is expected of them as participants (e.g., to disclose vulnerabilities, offer social support, thank colleagues at the end).

Studies of mutual aid groups like AA (e.g., Denzin, 1987: 118) have argued for the importance of ritual as “a symbolic bridge between the person and the group.” The ritual builds solidarity among participants and enacts group values (Borkman, 1999; Denzin, 1987). Descriptions of rituals in mutual aid groups suggest that they may be more finely scripted and institutionalized than the ritual performances we observed in our study. Yet, there are some similarities, including the reliance on storytelling and the progression toward social support that we saw in typical episodes. We suggest that this enables participants to experiment with alternative

ways of sharing and relating to others that might otherwise have appeared prohibited. The path anomalies observed suggest, however, that the tension playing out involves a precarious balance between social pressure and social support, as we see next.

Incremental progression in individual disclosure from facts to emotions. A second aspect of the observed practices, which we theorize helps to explain how and why participants are able to reveal vulnerability in work-related contexts, relates to the incremental nature of the social pressure applied during the individual disclosure phase. In contrast with mutual aid groups, where individuals typically develop a holistic story from the beginning without intervention from others (Denzin, 1987), for the vulnerability practices observed, it was usually through ongoing questioning that participants moved from facts to more emotionally charged accounts in which their vulnerabilities were revealed. We argue that this progression from “facts” to “emotions” legitimates disclosure, as long as the questioning remains nonjudgmental and empathetic while the story is enriched and its emotional implications for the storyteller are revealed. Although the context is different, this approach is common in recommendations surrounding clinical debriefing related to stressful events (Mitchell & Everly, 1996), where facilitators are urged to move successively from facts and thoughts to emotional disclosure. We suggest that none of this would be acceptable, however, without the third aspect, which enacts the transition from social pressure to social support: the set of practices we labeled “theorizing from the case.”

Multifaceted theorizing to seed social support and learning. Our analysis reveals the pivotal role of theorizing from the case within episodes of vulnerability practices, placed within the central circle of Figure 2 to signal its pivotal role in bridging from the individual disclosure phase to the communal reflection phase. We use the term “theorizing” to refer to a multifaceted set of practices for abstracting away from, generalizing, or reimagining the nature of the situation discussed. Theorizing plays two roles here. First, it enacts the interpretive function of a “holding environment” (Kahn, 2001) and enables containment of the difficult emotional experiences shared by the storyteller. This, in turn, seeds social support through validation and advice. Second, theorizing from the case turns the practice of revealing vulnerability into an opportunity to learn for all participants in the ritual (Kahn, 2001; Petriglieri & Petriglieri, 2020). Other collective settings where vulnerable individuals interact are known to involve

specific interpretive resources (e.g., the 12-step process of AA). We identified a rich collection of interpretive resources related to vulnerability practices in multidisciplinary settings.

Specifically, *boundary setting* involves stepping away from the events of the case to reflect on the theoretical limits of the professional role. This happened in our case when a storyteller appeared to be allowing a client’s stress, pain, or personal problems to intrude on their own well-being. Here, we see parallels with studies of efforts by professionals to manage and regulate interactions that involve them in clients’ emotional and personal concerns (Hayward & Tuckey, 2011; Heaphy, 2017). We noticed, in our study, that the facilitator (psychologist or senior physician) often led this theorizing practice, stimulating reflection among others. This reflected their professional expertise in therapeutic relations, and constituted part of the “liturgy” they brought to the ritual. Facilitators in different settings (e.g., faculty members in action-learning sets—see Corlett et al., 2021) might bring different forms of theoretical expertise that serve a similar purpose.

Categorizing is a different theorizing practice that locates the situation described by the storyteller in an abstract category that represents it, relating the specific phenomenon observed to a common experience. Our study reveals how abstraction away from the specific case can distance the storyteller from the situation that generated feelings of doubt and insecurity. Writers in the sociology of medical diagnosis (Jutel & Nettleton, 2011) have noted how *naming* a medical condition legitimizes an illness for a patient. A parallel phenomenon may occur for the storyteller in the context of vulnerability practices. Naming the type of patient case (or category of client for another professional practice; e.g., the “hyperactive” student, the child from a broken family) serves to collectivize the issue being discussed, making the experience relatable, and displacing responsibility away from the storyteller. Categorizing may be a particular expertise of higher-status members in a multidisciplinary team, who are likely to have distinctive ability in diagnosis.

Finally, the practice of *perspective shifting* involves reimagining the situation from the viewpoint of clients (or others involved in the situation discussed). The term “perspective taking” is often used to capture the cognitive phenomenon of viewing things from another’s perspective (Boland & Tenkasi, 1995; Galinsky, Ku & Wang, 2005) with positive effects on empathy and collaboration. We use the term “perspective shifting” to connote more than a cognitive shift, but, rather, an interactive approach in

which one group member represents the generic position of the “other,” drawing on a credible personal affinity with that role (as when a patient partner reflects the viewpoint of a client). The involvement of clients in vulnerability practices among professionals introduces experiential interpretive resources for theorizing that would otherwise be absent.

As we see, the multidisciplinary of the team (and its corresponding epistemic diversity) widens the range of interpretive resources available, providing complementary ways to make sense of particular cases. For example, while categorizing might involve a risk of “stereotyping” (Hilton & von Hippel, 1996) that underrecognizes client specificities, perspective shifting (notably by a patient partner) can nuance attempts to “blame the patient.” Participants may then draw on the interpretive resources available to construct “accounts” of what was happening in the storyteller’s interactions with a client, empathetically legitimating different views (Heaphy, 2017), and opening up pathways for social support and role-based learning. In other words, multidisciplinary settings offer unique opportunities for expanding understandings of role enactment.

In sum, theorizing from the case constitutes a pivotal set of practices that initiates the movement toward communality, providing colleagues with the resources needed to validate the storyteller’s account and offer nonjudgmental advice—key elements to contain difficult emotions and maintain a “good enough” holding environment. However, not all participants are necessarily “held” equally. The tension between social pressure and social support resurfaces in participants’ appraisals of the benefits and risks of vulnerability practices, as we discuss next.

Tensions Underpinning Participants’ Experiences of Vulnerability Practices

Research has emphasized the potential of vulnerability displays among professionals to create possibilities for social support (Corlett et al., 2019; Hibbert et al., 2022). This stream of research has presented vulnerability displays as broadly beneficial and rooted in relational interdependencies among human beings (Butler, 2016; Jordan, 2008). At first sight, our study of the ritualized nature of vulnerability practices aligns with these ideas. As illustrated by the feedback loop at the bottom of Figure 2, participants in our study often appraised vulnerability practices as beneficial because they offered comfort, reassurance, and opportunities for role-based learning. Our analysis of the ritual, and especially the pivotal role of theorizing practices,

helps to explain how these beneficial effects are generated.

Yet, in our Findings, and more so for some participants than others, vulnerability practices could also be experienced as risky. This is captured in the social discomfort associated with disclosing one’s role-based doubts and insecurities (especially to more senior participants). In the worst case, when theorizing is skipped (as in our under-validation path anomaly), this exposure might lead to experiences of devaluation or harm, as the setting fails to provide a good enough holding environment for the storyteller. Studies of vulnerability at work have either highlighted how professional norms discourage vulnerability display (Bourgoin & Harvey, 2018; Pratt et al., 2006) or have emphasized the positive effects of displaying vulnerability in specific contexts (Brown, 2015; Corlett et al., 2021; Hibbert et al., 2022). We nuance this by exploring how and when vulnerability practices may have unintended consequences for some participants. Indeed, revealing vulnerability involves risk, as the storyteller places their doubts and insecurities in the hands of the group, which may or may not provide empathy and validation to contain difficult emotions. Every new episode of vulnerability practices is a precarious undertaking fraught with risks and potentially unexpected effects for its participants. The ongoing tension between risks and benefits is represented by the arc balancing social pressure with social support in Figure 2.

Our findings suggest that two elements may shape the appraisal of benefits and risks associated with vulnerability practices for participants. The first element is experiential, expressed by the feedback loop at the bottom of Figure 2. As we showed, greater experience with the ritual tended to attenuate the social discomfort that participants experienced, constituting the setting as a “holding environment” for them. We suggest that the predictability of the ritual (Smith & Stewart, 2011), the incremental progression of individual disclosure from facts to emotions, and the multifaceted nature of theorizing that seeds social support and role-based learning, enable this to occur. When participants see those who reveal their vulnerabilities receiving social support, they are likely to become more willing to reveal vulnerability as well. However, an incident of obvious under-validation risks derailing this process, both for the storyteller and for colleagues.

Another element that can influence participants’ appraisals of the risks and benefits of engaging in vulnerability practices concerns power and authority dynamics (see the top of Figure 2), which we discuss in more detail in the next section.

Power and Authority Dynamics in Multidisciplinary Settings of Vulnerability Practices

The sequence of vulnerability practices we have conceptualized forms a ritual unfolding in a dedicated setting that allows for counter-institutional understandings to emerge (Chreim et al., 2020). Rituals are typically described as flattening institutionalized hierarchies and authority relations, contributing to a sense of comfort by enabling reciprocity among participants (Bell, 1992; Turner, 1969). In our study, however, we observed the enactment of role relations that were distinct from those prevailing outside it, implying not necessarily a flatter, but, rather, an alternate hierarchy. We identify four features of the setting that might shape power dynamics in the ritual: (1) the role of the facilitator, (2) degree of discretion to participate or not, (3) theorizing capability, and (4) position in institutionalized hierarchies (see online Appendix 4). We argue that some of these power dynamics may be commonly observed in settings not characterized by multidisciplinary while others are specific to it, or potentially reinforced by it.

A first aspect of the power dynamics that shape the setting where vulnerability practices are enacted concerns the *role of the facilitator*. In research on organizational rituals, power is seen as wielded mainly by a facilitator, or “ritual specialist” (Johnson et al., 2010). In our case, the facilitator was always a senior member of the team—a psychologist or a physician, the two highest-status disciplines in this context. The facilitator was a clear figure of authority who could call on a participant to act as storyteller, ask questions, or lead theorizing from the case. The facilitator’s ability to play this role depends on the legitimacy conferred on them by other participants, which cannot be taken for granted. Thus, status within the professional hierarchy helps to legitimize their role in managing the orderly conduct of vulnerability practices. Kahn (2001) points out that supervisors can have a major role in creating and maintaining holding environments that instill a sense of comfort in employees, enabling them to surface and work through their anxiety. Kahn (2001: 271), however, also indicates that “holding environments are fragile,” and points to boundaries and power differences that mark relations among participants in such environments. Indeed, some research has provided evidence of members’ hesitation to participate in these settings (Corlett et al., 2021; Pinder et al., 2006; Sweeney et al., 2020), and our study shows how multiple incursions of power and authority might mitigate people’s willingness to display vulnerability.

Thus, a second important element that we theorize as influencing the power dynamics is the degree of *discretion to participate or not*. Participation may not always be completely voluntary. In our case, some participants were expected, more than others, to participate in the activity because it constituted an educational requirement (for residents) or an opportunity to train others (for senior physicians). The social pressure to make oneself vulnerable during the individual disclosure phase will thus vary in intensity depending on a person’s degree of discretion to respond when the facilitator invites them to share a story of hardship at work. Refusing to do so may be seen as breaking conventions of acceptable behavior. The multidisciplinary nature of a group may enhance social pressure if some members feel constrained to participate, but are reluctant to reveal their experiences in front of certain others (e.g., the patient partner, in our case).

A third aspect of the power dynamics relates to *theorizing capability*. Indeed, engaging in theorizing practices implies authority to define situations and prescribe solutions, which may not be evenly distributed. Of the three theorizing practices observed, the facilitator’s expertise seemed most associated with *boundary setting* because this language is grounded in psychotherapy training. *Categorizing* is potentially available to all, but long experience (related to seniority) and higher disciplinary status could make some interlocutors more credible than others. Finally, *perspective shifting* was mainly the province of those who could most genuinely represent the clients’ experiential knowledge (i.e., patient partners). Power may also manifest when theorizing involves invoking esoteric language that is accessible to some and not to others (Vaara & Whittle, 2022). As we illustrated in the findings, this form of obfuscation marginalizes participants to whom the language is unfamiliar, which can be particularly evident in multidisciplinary settings.

Finally, a fourth aspect of power dynamics in these settings relates to *position in institutionalized hierarchies* within and across professions (Abbott, 1988). Multidisciplinary is associated with hierarchies that may be difficult to downplay when imported into a new setting. They are embedded in relationships among participants and may be more or less overt; they may manifest intentionally, or seep imperceptibly into the setting where vulnerability practices occur.

On the one hand, our study shows various dynamics that help reduce manifestations of power, such as willingness by people in positions of hierarchical

authority to reveal vulnerability. We found that participants in lower hierarchical positions could also assert authority by advocating for perspectives grounded in a specialized base of expertise recognized and valued by participants. This brings us back to the surprisingly influential role of patient partners in our setting. In several episodes, we observed a patient partner suddenly taking over the mic to forcefully advocate for colleagues to change their professional practices, or challenge their assumptions from a patient's experiential perspective (Beresford, 2019). These disruptive interventions appeared to be valued by their professional colleagues. Patient partners also tended to make themselves vulnerable with unique ease and authority, based on their experiential knowledge of living with chronic illness and care. Patient partners could subvert professional epistemologies with clients' experiential understandings (Dumez & L'Espérance, 2024), promoting testimonial modes of knowledge validation (Arminen, 2004; Borkman, 1999) that might otherwise be considered illegitimate.

On the other hand, our study also showed how other members of the team, and especially those usually considered lower in the professional hierarchy, resisted vulnerability practices. Indeed, despite social pressure, participants may refrain from making themselves vulnerable. As Bell (1992: 222) indicates, "ritualized practices, of necessity, require the external consent of participants while simultaneously tolerating a fair degree of internal resistance." These findings point to the limits of rituals of vulnerability practices. Questioning the facilitator's authority casts doubt on the legitimacy of the ritual, and those who questioned it in our study did indeed indicate that participation was not effective for them (e.g., seeing themselves cast as incompetent). These instances reveal the precarity of the holding environment enacted by the team here.

Boundary Conditions and Future Research

The model we develop is most likely to apply when vulnerability practices take place outside of the regular work activities, and benefit from the predictability that ritualization provides members of the group engaging in these practices. There are other contexts, however, where vulnerability displays are embedded in the everyday activities of workers. Examples include peer support work related to psychosocial distress, homelessness, and sexual abuse (Barker & Maguire, 2017; Zilber, 2002). In such settings, displays of vulnerability are central to the regular exchanges that occur between workers and the clients they support. The single case design

of our study was explicitly focused on vulnerability practices in a context where doubts and insecurities about professional roles were the key focus of attention. Thus, future research may adopt a multi-case approach to compare various contexts, processes, and dynamics of vulnerability practices. Further, as the activity we studied was conducted entirely online during our period of observation, analyzing similarities and differences in how professionals engage in vulnerability practices online versus in person might uncover novel insights.

Also, the meetings we observed included people of various disciplines. While this condition can offer a rich array of interpretive resources for workers to learn about how different vulnerabilities are dealt with across occupations, they may also make the exchanges appear intimidating or invalidating for some participants. Thus, while our study sheds new light on power dynamics related to disciplinary status inequalities in reflexive social settings, it leaves some questions unanswered. Future research might examine other groups that include diverse disciplinary and seniority statuses to understand how it might be possible to level the playing field and enhance the benefits of vulnerability practices for those involved. For example, one might ask how rotating the facilitator role among participants of different disciplinary statuses, or organizing groups with participants who do not otherwise work together, might induce shifts in power dynamics and appraisals of benefits and risks.

CONCLUSION

The literature has documented the challenges that professionals encounter in sustaining an illusion of invulnerability. Moreover, some scholars have recognized the benefits of sharing vulnerabilities with colleagues to create opportunities for validation and social support at work. In this paper, we reveal *how* this might be achieved through a ritualized sequence of practices in a setting dedicated to that purpose. Yet, while feelings of vulnerability may be pervasive in professional contexts, settings to share and learn from vulnerability are rare and participating in them is often not considered legitimate "work." Nonetheless, the implicit value of this setting motivated the participants we observed to dedicate up to three hours of their busy weekly schedules, which served, according to one, as a "lifeboat," enabling them to deal with vulnerability at work and reshape their expectations about how they and their colleagues should perform their work roles. We argue that

recognizing sustained engagement in vulnerability practices as “real work” is not only a contribution to theory, but a practical one as well. Indeed, this recognition may help practitioners (including academics) to value such settings as potential holding environments (or “lifeboats”) to cope with the pervasive doubts and insecurities we all experience in our daily work.

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